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*Institute of Politics Policy Program
Health Policy Team*

**Combatting Medical Debt
in Massachusetts:
Expanding Care Access and
Regulating Patient Costs**



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Executive Summary

The Commonwealth of Massachusetts is recognized for having one of the strongest healthcare systems in the United States, combining a high concentration of healthcare providers, quality services, and near-universal medical insurance. However, these advantages of the Massachusetts healthcare system are not without drawbacks, most notably the high cost of care patients within the state face. As a result of these costs, more than one in eight Massachusetts residents has medical debt, including hundreds of thousands with health insurance. Although the state has implemented programs that seek to reduce medical costs faced by uninsured and underinsured patients, such as the Health Safety Net, these programs, in their current form, are unable to meet the cost-relief needs of Massachusetts residents.

At present, Massachusetts plays a limited role in regulating the costs borne by residents seeking medical care, often leaving these patients to face high copays and coinsurance costs, or, in the case of uninsured patients, highly inflated hospital, physician, and pharmaceutical charges. As a result, patients unable to cover their own bills must take on medical debt, unless they qualify for—and are able to receive support from—one of the state’s programs or receive financial aid or a fee waiver from their provider. Thus, a comprehensive response to Massachusetts’ medical debt crisis requires addressing both the cost of care delivery itself—including medical procedures, pharmaceuticals, and hospital care—and state programs and regulations that are currently insufficient to ameliorate and protect patients from medical debt.

Although recent approaches have begun addressing the drivers of medical debt in Massachusetts, further policy development and innovation are necessary. To ensure that these processes reduce medical debt in a holistic and equitable manner, both the Massachusetts legislature and executive departments play an active role in addressing its causes. The Massachusetts legislature must lead efforts to strengthen existing state-run financial assistance programs, improve financial transparency and cost reporting requirements by health practitioners and pharmaceutical and medical device manufacturers, and increase the accessibility of practitioner financial aid programs to patients. In tandem, state executive agencies, primarily led by the Executive Office of Health and Human Services, should utilize their regulatory authority to ensure the functionality of state relief for medical debt, engage in partnerships within the state and nationally to improve care delivery and reduce existing debt, and engage with federal regulators to better realize the state’s equity and care delivery goals. Together, this multi-pronged policy approach represents a meaningful improvement in Massachusetts’ commitment to ensuring that the cost of medical care and burden of medical debt do not prevent patients from pursuing and receiving the care they require.

Background Information

Drivers of Medical Debt in Massachusetts

Massachusetts has been ranked as having the best healthcare system in America by many metrics. More than 98% of residents have some kind of health insurance, along with internationally-renowned hospitals located throughout the state, contributing to a high quality of care.¹ However, these impressive statistics hide a looming crisis within the state. Medical debt is currently a devastating burden across Massachusetts, affecting residents across boundaries of age, race, gender, ethnicity, and geographical location.² Despite the fact that 98.3% of residents have a form of health insurance, more than 40% of them stated that they struggle to pay health costs,³ which is especially concerning considering that the state has the fourth-highest average cost of care per patient and the second-highest family health insurance premium out of all fifty states.⁴ These disparities also disproportionately impact rural patients, low-income communities, immigrants, and people of color across Massachusetts, with many of them unable to benefit from the coverage that has been codified into law as a right for every Massachusetts resident. Thus, it is essential to better understand how this issue has manifested itself within the Massachusetts healthcare system.

¹ “Mass. Insured Struggling With Health Care Access, Costs | State House News Service.” 2024.

MASSACHUSETTS CENTER FOR HEALTH INFORMATION AND ANALYSIS.

<https://www.chiamass.gov/mass-insured-struggling-with-health-care-access-costs-state-house-news-service>.

² “Health Care Affordability for Residents and Their Families.” 2024. MASSACHUSETTS CENTER FOR HEALTH INFORMATION AND ANALYSIS.

<https://www.chiamass.gov/assets/docs/r/survey/mhis-2023/MHIS-2023-04-Health-Care-Affordability.pdf>.

³ “Mass. Insured Struggling With Health Care Access, Costs | State House News Service.” 2024.

MASSACHUSETTS CENTER FOR HEALTH

INFORMATION AND ANALYSIS.

<https://www.chiamass.gov/mass-insured-struggling-with-health-care-access-costs-state-house-news-service>.

⁴ “Healthcare Costs: Where does your state rank?” n.d. Umbra Health Advocacy. Accessed October 6, 2025.

<https://umbrahealthadvocacy.com/where-does-your-state-rank-in-healthcare-costs-the-most-and-least-expensive-states-for-care/>.

Across the United States, and in recent years, medical debt has become a primary issue in many families' lives, despite over 90% of U.S. families possessing some form of health insurance.⁵ Naturally, for individuals with chronic illnesses and other ailments that require more frequent medical assistance, these debts can accumulate at a faster rate. For example, individuals battling cancer are susceptible to accumulating higher levels of debt than people without cancer. This disparity has a variety of negative impacts, with Americans in medical debt often resorting to not allocating sufficient funds to other life necessities such as food, clothing, and basic utilities.⁶ The same difficulties apply to individuals of greater age, with middle-aged adults incurring disproportionate out-of-pocket healthcare expenses compared to other demographics. 10% of adults ages 50 to 64 report having medical debt, compared to 6% of those ages 65 to 79.⁷ For a state like Massachusetts, where the median age is 40.3 and 62% of residents are between the ages of 18 and 64, this is a particularly pressing issue.⁸

Socioeconomic Status and Systemic Barriers

Across the country, as well as at the state level, socioeconomic status and demographics have a strong correlation with the accumulation of medical debt. Although Massachusetts has near-universal health insurance coverage, with 98.3% of residents reporting some form of insurance, residents continued to report challenges in paying for needed healthcare in 2023. Nearly half of residents (41.3%) reported that they or their families had any health care affordability issues, and 16.5% of residents reported multiple affordability issues in their

⁵ Shameek Rakshit et al., "The Burden of Medical Debt in the United States," Peterson-KFF Health System Tracker, February 12, 2024, <https://www.healthsystemtracker.org/brief/the-burden-of-medical-debt-in-the-united-states/>.

⁶ Kffalexm, "Health Care Debt in the U.S.: The Broad Consequences of Medical and Dental Bills," KFF, August 9, 2025, <https://www.kff.org/health-costs/kff-health-care-debt-survey/#c7f7c77b-dec4-4baf-bed1-78e3522b4f46>.

⁷ Shameek Rakshit et al., "The Burden of Medical Debt in the United States," Peterson-KFF Health System Tracker, February 12, 2024,

⁸ "Census Profile: Massachusetts," Census Reporter, accessed October 6, 2025, <https://censusreporter.org/profiles/04000US25-massachusetts/>.

families.⁹ Nearly half of non-Hispanic Black residents (48.7%) and three in five Hispanic residents (58.2%) reported at least one affordability issue for their families compared with 39.2% of non-Hispanic White residents.¹⁰

Additionally, most surveyed residents had problems paying for medical tests or procedural bills (50.6%).¹¹ In 2023, one in eight (12.8%) Massachusetts residents reported that their family held medical debt, or family medical bills that are being paid over time. Non-Hispanic Black residents (20.0%), as well as residents with someone in their family who went without insurance in the past 12 months (29.4%), were most likely to report family medical debt.¹² Overall, there is a clear disproportionate impact on Black and Hispanic families regarding the medical debt they face. This could have several reasons for occurring, including the residential segregation implemented by the HOLC after WWII, systemic factors like low income, and limited access to affordable health insurance within communities of color. In addition, in many communities where people of color live, poverty rates are disproportionately high, levels of pollution and crime are elevated, and green spaces are few and far between, which are all key contributors to health disparities.¹³ These factors all generate an uptick in medical injuries within communities of color. This uptick, coupled with the fact that, on average, Black and Latino families are less likely to be insured as opposed to their white counterparts, heralds a populace with higher rates of accumulated medical debt.

⁹ Findings from the 2023 Massachusetts health insurance ..., June 2024, <https://www.chiamass.gov/assets/docs/r/survey/mhis-2023/MHIS-2023-04-Health-Care-Affordability.pdf>.

¹⁰ Findings from the 2023 Massachusetts health insurance ..., June 2024, <https://www.chiamass.gov/assets/docs/r/survey/mhis-2023/MHIS-2023-04-Health-Care-Affordability.pdf>.

¹¹ Findings from the 2023 Massachusetts health insurance ..., June 2024, <https://www.chiamass.gov/assets/docs/r/survey/mhis-2023/MHIS-2023-04-Health-Care-Affordability.pdf>.

¹² Findings from the 2023 Massachusetts health insurance ..., June 2024, <https://www.chiamass.gov/assets/docs/r/survey/mhis-2023/MHIS-2023-04-Health-Care-Affordability.pdf>.

¹³ Advancing Racial Equity in U.S. health care: State disparities | commonwealth fund, April 18, 2024, <https://www.commonwealthfund.org/publications/fund-reports/2024/apr/advancing-racial-equity-us-health-care>.

Inaccessible Financial Aid Assistance

Generally, the price of healthcare in Massachusetts is among the highest in the country, primarily due to the state's high cost of living. Massachusetts has the second-highest family insurance premiums in the country, and some hospitals set prices significantly above those Medicare pays for the same service because its health systems hold immense negotiating power due to market consolidation. Despite the existence of policies such as the federal financial assistance policy, which requires all tax-exempt hospitals to establish a written policy offering free or discounted care to qualifying patients, many low-income and chronically ill individuals still face debilitating medical costs due to the broad discretion hospitals have in designing their financial assistance programs. Additionally, the details of these programs are often inaccessible to patients of lower socioeconomic status due to inadequate education and poor communication on the part of the hospitals. Applying for financial assistance in Massachusetts also proves to be particularly burdensome because each Massachusetts hospital has a unique application for financial aid, unlike other states such as Maryland and Colorado, which mandate a standardized financial assistance application.¹⁴ Additionally, unlike states such as Oregon, Massachusetts hospitals are not obligated to provide detailed annual data on financial assistance provided, accounts sent to collections, and lawsuits filed over medical debt, ensuring transparency and accountability.

Commercial Spending in Relation to Debt Accumulation

Commercial spending, in general, is also positively correlated with health care costs. Massachusetts has one of the highest costs of living in the U.S. and the second-highest

¹⁴ David Velasquez, “Medical Debt Relief Isn’t Just about Policy. It’s about People,” Cognoscenti, May 1, 2025, <https://www.wbur.org/cognoscenti/2025/05/01/medical-debt-massachusetts-federal-financial-assistance-david-velasquez>.

commercial premiums in the country; the cost of living exceeds the average income for families across the state. If premium growth continues at the current rate of 7% rather than at the benchmark rate of 3.6%, take-home pay for an average Massachusetts family in 2030 would be reduced by \$600 per month.¹⁵ As health care spending grows as a portion of household income, more and more families incur medical debt and avoid using needed care. These rates become particularly dire when health care premiums and out-of-pocket spending reach 25% of total income—a reality for 41% of Hispanic families and 26% of Black families in Massachusetts in 2023, compared to 9% of white families.

Digital Divide and Rural Disparities

One key contributor to the exacerbation of medical debt in Massachusetts is the long-standing technological disparities between cities and suburban areas in comparison to rural counties and low-income communities. Healthcare has been steadily digitizing over the past decade, especially in the wake of the COVID-19 pandemic, with hospitals heavily emphasizing the necessity of telemedicine.¹⁶ Furthermore, artificial intelligence's rapid rise over the past two years has revolutionized the way healthcare systems operate.¹⁷ However, this trend has almost exclusively been concentrated in big cities, like Boston and Worcester, while rural areas like Berkshire and Franklin counties have lagged behind. This is largely due to the lack of healthcare professionals who can assist with technology and other necessary resources in these regions,

¹⁵ Health care spending grew faster than inflation, labor costs, and income, exacerbating affordability challenges for Massachusetts residents | massachusetts health policy commission, accessed October 7, 2025, <https://masshpc.gov/news/press-release/health-care-spending-grew-faster-inflation-labor-costs-and-income-exacerbating>.

¹⁶ Holmgren, A. J., McBride, S., Gale, B., et al. "Technology as a Tool for Improving Patient Safety." *PSNet*. Rockville, MD: Agency for Healthcare Research and Quality, U.S. Department of Health and Human Services, 2023. <https://psnet.ahrq.gov/>.

¹⁷ Rahman, M. A., E. Victoros, J. Ernest, R. Davis, Y. Shanjana, and M. R. Islam. "Impact of Artificial Intelligence (AI) Technology in Healthcare Sector: A Critical Evaluation of Both Sides of the Coin." *Clinical Pathology* 17 (January 22, 2024): 2632010X241226887. <https://doi.org/10.1177/2632010X241226887>.

especially differences in broadband access. Without it, it is extremely difficult for rural residents to access their financial information as well as any documents required for their health insurance, considering that more than 86% of hospitals nationwide have shifted to an online-based health finance system.¹⁸ For example, in Middlesex County, which is one of America's most populous counties and is almost entirely urban and suburban, 71% of residents have stable broadband connections.¹⁹ In contrast, in Franklin County, fewer than half of the population has broadband access, while in Berkshire County, just 39% of residents are connected to broadband.⁷

This disparity transcends a rural-urban division, further being separated across wealth lines. Specifically, less affluent regions of Massachusetts tend to have more limited broadband access, with Hampden County, which has the lowest average income for a county in Massachusetts,²⁰ having 56% of residents able to access broadband, while in Nantucket County, the state's wealthiest, 100% of residents have stable broadband access. This severe gap contributes to the medical debt crisis by impeding access in low-income and rural counties to essential information for banking and understanding health insurance. In particular, many residents of these areas depend on public Wi-Fi or mobile connections due to the significant lack of broadband coverage. This digital access inequity means that patients who can't access online systems miss deadlines, fail to complete applications, and misunderstand bills, contributing to and exacerbating unnecessary debt. The result is an unassuming but devastating barrier; it's a system designed to encourage digital fluency, yet inadvertently excludes those it's most necessary for.

¹⁸ "Digital by Default - 85% of Americans Have Online Health Insurance Accounts." 2025. Flexpa. <https://www.flexpa.com/blog/digital-by-default>.

¹⁹ King, Ledyard. 2021. "In Massachusetts, many still lack broadband access." The Telegram. <https://www.telegram.com/story/news/2021/07/07/gda-broadband-local-ma-nteg/47205641/?gnt-cfr=1&gca-cat=p&gca-uir=true&gca-epti=z114134e008100v114134b0046xxd004665&gca-ft=135&gca-ds=sophi>.

²⁰ Pavone, Sean. 2025. "Highest-earning counties in Massachusetts." Stacker. <https://stacker.com/stories/massachusetts/highest-earning-counties-massachusetts>.

Geographic Health-Cost Divide

In Massachusetts, geography proves to be a major barrier and factor when it comes to medical debt, creating significant health-cost divides. Within the city of Boston, there are substantial geographic disparities among neighborhoods. In neighborhoods of predominantly Black and Latinx residents and of lower median incomes, such as East Boston, Dorchester, Roxbury, and Mattapan, residents are nearly two times as likely to be uninsured compared to the rest of the city. Derrick Marshall, owner of Namel’s Barber Salon in the heart of Dorchester, noted that for most residents, “health insurance... is a consideration that comes later in life.”²¹ The burden of these health-cost divides weighs heavily on certain neighborhoods, with half of the residents of Roxbury and Mattapan having “subprime” credit scores (580–619, compared to a “prime” credit score of 660–719), while only 8 percent of Newton and Beacon Hill residents have such scores.²² Similarly, approximately 1 in 3 residents of Roxbury and Mattapan have dealt with debt collection, nearly 7 times the rate of Newton or Beacon Hill.²³

However, these disparities not only manifest at the city level but also exist at the regional level. Residents of the South Coast, including the cities of Fall River and New Bedford, experience the highest rate of medical debt across the entirety of Massachusetts at 27.2 percent.²⁴ Those in the South Coast reported having issues paying medical bills at far higher rates than other urban hubs like Boston, with nearly 20 percent of residents noting struggles, compared to

²¹ Bedford, Tori. “Health Connector Workers Target Underserved Communities as Open Enrollment Approaches.” GBH. October 28, 2021. <https://www.wgbh.org/news/local/2021-10-28/health-connector-workers-target-underserved-communities-as-open-enrollment-approaches>.

²² Reserve Bank, “Federal Reserve Bank of Boston,” Federal Reserve Bank of Boston, June 27, 2018, <https://www.bostonfed.org/publications/community-development-issue-briefs/2018/the-concentration-of-financial-disadvantage.aspx>.

²³ Reserve Bank, “Federal Reserve Bank of Boston.”

²⁴ “South Coast Residents Struggle to Get Health Care, Insurance,” Chiamass.gov, 2025, <https://www.chiamass.gov/south-coast-residents-struggle-to-get-health-care-insurance?>

11.5 percent in Boston and the state average of 12.5 percent.²⁵ Despite similar insurance coverage in South Coast when compared to the state average (89.4 percent and 93.3 percent, respectively), South Coast residents are far more likely to delay or forgo treatment, with 31.3 percent of residents frequently skipping care due to cost.²⁶

These evident gaps are further exacerbated by the disparities in accessibility depending on the region. While residents in downtown Boston can access a hospital by walking a mere few blocks, and there are 23 other acute care hospitals within a 30-minute radius of Tufts Medical Center, in the more rural Berkshire County, there are no other acute care hospitals within 30 minutes of Berkshire Medical Center.²⁷ As a state, Massachusetts has 46 “medically underserved areas”; the majority of these areas have had this title for nearly three decades, speaking to the lack of effective improvements and policies that have been enacted.²⁸ A resident coming from a more rural part of Massachusetts is thus burdened with finding transit, child care, access to technology, and mental energy in order to merely access healthcare, contributing to the already uneven medical debt they face.

Immigration Status and Language Barriers

These geographic disparities are further worsened by the persistent language barriers and unfriendliness towards immigrants that the healthcare system possesses. In the aforementioned South Coast region, New Bedford, in particular, over 40.9 percent of the population is made up

²⁵ David Seltz, “Emerging Challenges in Health Care Access, Affordability, and Workforce” (Massachusetts Health Policy Commission, April 2024),

https://masshpc.gov/sites/default/files/2024-05/20240424_Emerging-Challenges-in-Southeastern-MA.pdf.

²⁶ Chiamass.gov, “*South Coast Residents Struggle to Get Health Care, Insurance.*”

²⁷ Jennifer Smith, “Red Lights on the Way to Health Care,” Commonwealth Beacon, September 5, 2025, <https://commonwealthbeacon.org/health-care/red-lights-on-the-way-to-health-care/>.

²⁸ Smith, “*Red Lights on the Way to Health Care.*”

of immigrants,²⁹ with upwards of 10 percent of the residents being undocumented immigrants.³⁰

As a result, the New Bedford Health Department has not only found that their residents have been disproportionately affected by COVID-19, the opioid epidemic, and homelessness, but that their Hispanic population, amongst other historically underserved communities, have especially poor health outcomes.³¹ This is further intensified by the language barriers. For example, in New Bedford, 70% of patients who utilize the central pediatric practice are unable to speak English, and thus also have a hard time trusting any formal, organized business, like a hospital.³²

Language barriers thus cause important factors, including treatment, duration, etc., to become lost in translation, again worsening the issue of medical debt and treatment for immigrants.

Because immigrants have a harder time receiving coverage, with undocumented immigrants being ineligible for Medicaid and lawfully present immigrants often having to wait five years before enrolling³³ and many immigrants in New Bedford, for example, holding more transient occupations without reliable healthcare coverage (landscapers, construction workers, cranberry harvesters, cleaners), immigrants of Massachusetts are disproportionately impacted by medical debt.³⁴

High Deductible Health Plans

In a study of the consequences of high deductible private insurance programs, Rabin and peers found that medical debt was greater among those with any kind of high deductible

²⁹ “Cultural Communities - New Bedford Whaling Museum,” New Bedford Whaling Museum, March 12, 2021, <https://www.whalingmuseum.org/research/research-resources/regional-history/cultural-communities/>.

³⁰ “Putting Community Health Front and Center in Massachusetts,” Partners In Health, 2023, <http://pih.org/article/putting-community-health-front-and-center-massachusetts>.

³¹ Partners In Health, “*Putting Community Health Front and Center in Massachusetts*.”

³² Chiamass.gov, “*South Coast Residents Struggle to Get Health Care, Insurance*.”

³³ kffsamanthaa, “5 Key Facts about Immigrants and Medicaid | KFF,” KFF, February 19, 2025, <https://www.kff.org/racial-equity-and-health-policy/5-key-facts-about-immigrants-and-medicaid/>

³⁴ Grace Ferguson, “How Mass Deportations Would Affect New Bedford’s Economy,” The New Bedford Light, February 11, 2025, <https://newbedfordlight.org/how-mass-deportations-would-affect-new-bedfords-economy/>.

insurance plan. Additionally, more vulnerable populations (e.g., low-income, marginalized, treatable chronic disease) experienced more severe debt.³⁵ Out of 62,087 participants, 13.0% were enrolled in a High-Deductible Health Plan (HDHP) with a Savings Option (SO) plan. This specific plan is a form of employer-provided insurance that actually increases deductible costs—from 2010 to 2016, average deductibles increased from \$991 to \$1,478. Additionally, 22.9% of respondents were registered under an HDHP with no SO. Of those with an HDHP with no SO, 17.1% reported medical debt; the association between being registered under this insurance plan and procuring medical debt was found to be statistically significant. Moreover, 11.8% of individuals enrolled in an HDHP with no SO reported deferring needed medical care. Among those enrolled in HDHPs with SO, 9% of respondents reported deferring needed medical care, while 12.6% had some form of medical debt. These figures are in comparison to only 10.4% and 6.1% of individuals registered under a no or low deductible plan experiencing medical debt and deferring medical care, respectively.

The long-term consequences of high deductibles and associated medical debt are clear, with the aforementioned study finding that for those in high-deductible programs experiencing high levels of medical debt, “deferred medical care [was] 6 fold,” with the highest rates seen among those with treatable chronic diseases.³⁶ In the case of diabetes, Brown and peers documented that low-income adults younger than 65 enrolled in higher deductible health plans attended significantly fewer “checkups, primary care physician visits, and specialty visits”, contributing to adverse health outcomes.

³⁵ Rabin et al., “Understanding the ACA Higher Deductibles and Medical Debt Cause Those Most Vulnerable to Defer Needed Care,” *Journal of Health Care for the Poor and Underserved* 31, (2020): 424-440.

³⁶ Rabin et al., “Understanding the ACA,” 424.

Redlining as a fundamental cause of unequal healthcare access in MA

It can be said that residential segregation is a fundamental cause of racial differences in socioeconomic status (SES), due to unequal access to employment, education, and healthcare.³⁷ In 1938, the Home Owners' Loan Corporation (HOLC), a government-sponsored agency, created color-coded maps of cities across the U.S., green for "A" (best), blue for "B" (still desirable), yellow for "C" (definitely declining), and red for "D" (hazardous). This initiative was part of a New Deal program intended to stabilize the housing market during the Great Depression by assessing mortgage risks. However, what it actually created were prejudiced assumptions about property value based on the racial and ethnic makeup of neighborhoods. As a result, Black, Latino, and immigrant neighborhoods were often colored red, or "redlined."³⁸

As a result, people in these neighborhoods were denied bank loans for home purchases and improvements, making it nearly impossible to buy homes and accumulate wealth. Redlined areas also saw less investment due to being labeled as "risky." Consequently, these neighborhoods were starved of community improvements and infrastructure, including well-funded hospitals and clinics and green spaces vital for health, while also facing hazardous environmental pollutants and limited access to healthy food options. This divestment led to overall low property values and less appreciation.

This governmental manifestation of racism is still evident in modern-day Massachusetts, particularly in its most populated and urban cities like Boston, where neighborhoods were historically color-coded through redlining. Areas impacted by redlining include Roxbury, the

³⁷ Williams, David R, and Chiquita Collins. "Racial Residential Segregation: A Fundamental Cause of Racial Disparities in Health." Public Health Reports 116, no. 5 (2001): 404–16.
[https://doi.org/10.1016/S0033-3549\(04\)50068-7](https://doi.org/10.1016/S0033-3549(04)50068-7).

³⁸ Acevedo-Garcia, Dolores. "Redlining and Present-Day Neighborhood Opportunity in the Boston Area." [https://www.Diversitydatakids.Org](https://www.diversitydatakids.org).
<https://www.diversitydatakids.org/research-library/data-visualization/redlining-and-present-day-neighborhood-opportunity-boston-area>.

South End, North End, West End, East Boston, Charlestown, and parts of South Boston and Dorchester. Additional redlined areas extended into the eastern parts of Cambridge and Somerville, as well as sections of Revere, Chelsea, Everett, and Malden.³⁹

When comparing these historically redlined neighborhoods to 2024 data on healthcare access in Boston, stark disparities remain. East Boston, for example, has the highest adult uninsurance rate by neighborhood at 10.8%, while Fenway has the lowest at just 0.5%. In terms of MassHealth enrollment, a state-run insurance program for individuals below the federal poverty line, Dorchester and East Boston rank the highest, with 73.9% and 65.9% enrollment rates, respectively.

Furthermore, these same neighborhoods, including Roxbury, have some of the lowest numbers of healthcare facilities, averaging just six facilities each. In contrast, Back Bay, a historically affluent area, has the highest number with 42 healthcare facilities. These statistics highlight the ongoing effects of systemic racism in healthcare access tied to historical redlining practices.⁴⁰

Existing Approaches to Debt Relief in Massachusetts

Health Safety Net and Funding Shortfalls

Massachusetts, like every other US state, houses constituents faced with crippling medical debt. In the contiguous United States, however, Massachusetts is home to the citizenry

³⁹ Acevedo-Garcia, Dolores. “Redlining and Present-Day Neighborhood Opportunity in the Boston Area.” <https://www.diversitydatakids.org/research-library/data-visualization/redlining-and-present-day-neighborhood-opportunity-boston-area>.

⁴⁰ Boston.Gov. “Health of Boston 2024.” https://www.boston.gov/sites/default/files/file/2024/06/HOB_Access%20to%20Care_2024_Final.pdf.

with the lowest percentage of individuals holding medical debt.⁴¹ While government programs aimed at offering assistance to those falling below the poverty line are quite abundant in Massachusetts—Health Safety Net (HSN) and Massachusetts Charity Care being the two most prolific—these programs are quickly headed toward record-level budget shortfalls.^{42,43} Despite the dwindling budget of debt relief programs, the percentage of individuals who now rely on HSN to field medical debt who live among the lowest income level has risen by 20 percent in the past year,⁴⁴ highlighting a key issue facing Massachusetts residents: an increasing number of individuals facing medical debt are becoming more impoverished, but the social programs aimed at mitigating medical debt are running out of essential funding to maintain their effectiveness.

If the number of people living at the lowest poverty level who rely on HSN increases in the years to come (as recent trends indicate they may),⁴⁵ fewer debt-relief opportunities will be available to those individuals. With national healthcare costs rising with increasing inflation,⁴⁶ Massachusetts will be no exception, for rising healthcare costs will only exacerbate the funding shortfalls of the HSN, leaving an increasing proportion of the most vulnerable Massachusetts residents without means to manage their medical debt.

⁴¹ Shameek Rakshit et al., “The Burden of Medical Debt in the United States,” Peterson-KFF Health System Tracker, February 12, 2024, <https://www.healthsystemtracker.org/brief/the-burden-of-medical-debt-in-the-united-states/#Share%20of%20adults%20who%20have%20medical%20debt,%20by%20health%20status%20and%20disability%20status,%202021>.

⁴² John LoDico, “Long-Term Care Report; New Safety Net Numbers,” Massachusetts Health & Hospital Association, August 11, 2025, <https://www.mhalink.org/mondayreport/long-term-care-report-new-safety-net-numbers/#1>.

⁴³ “Massachusetts’ Charity Care Fund Is Running on Empty,” Healthcare for All, March 6, 2025, <https://hcfama.org/massachusetts-charity-care-fund-is-running-on-empty-boston-globe-march-6-2025/>.

⁴⁴ Jeffrey Shapiro, “2025 Annual Report on MassHealth and the Health Safety Net: An Analysis of Health Safety Net Parents, Providers, and Services,” Office of the Inspector General of Massachusetts, March 3, 2025, <https://www.mass.gov/news/oig-releases-annual-report-on-public-healthcare>

⁴⁵ Shapiro, “2025 Annual Report on MassHealth and the Health Safety Net: An Analysis of Health Safety Net Parents, Providers, and Services”

⁴⁶ Shameek Rakshit et al., “How Does Medical Inflation Compare to Inflation in the Rest of the Economy?,” Peterson-KFF Health System Tracker, August 2, 2024, [https://www.healthsystemtracker.org/brief/how-does-medical-inflation-compare-to-inflation-in-the-rest-of-the-economy/#Cumulative%20percent%20change%20in%20Consumer%20Price%20Index%20for%20All%20Urban%20Consumers%20\(CPI-U\)%20for%20medical%20care%20and%20for%20all%20goods%20and%20services,%20January%202000%20-%20June%202024](https://www.healthsystemtracker.org/brief/how-does-medical-inflation-compare-to-inflation-in-the-rest-of-the-economy/#Cumulative%20percent%20change%20in%20Consumer%20Price%20Index%20for%20All%20Urban%20Consumers%20(CPI-U)%20for%20medical%20care%20and%20for%20all%20goods%20and%20services,%20January%202000%20-%20June%202024).

Access to Debt Relief Programs throughout Massachusetts

Programs like HSN are accessible to all eligible Massachusetts residents, and the processes for relieving medical debt are standardized without respect to a resident's region of the state.⁴⁷ However, hospital-specific policies are a major source of variation in Massachusetts residents' access to medical debt relief programs. Hospital-specific programs aimed at helping economically vulnerable patients are often referred to as "charity care" programs. While some states have enacted policies mandating charity care at each hospital, Massachusetts is not among these states.⁴⁸

Rather, only non-profit hospitals are legally required to provide charity care to Massachusetts residents.⁴⁹ As a result of the state's failure to mandate equitable charity care funding and the vast geographical landscapes that compose Massachusetts and the populations that reside in them, local charity care programs vary extensively in accessibility and capacity to meet patient needs. Individuals residing in the most populous parts of Massachusetts, such as Boston, for example, are surrounded by a plethora of hospitals funded by both state and federal grants and therefore possess the resources to provide robust debt forgiveness programs. Conversely, residents living far from the biotechnology capital in areas such as western Massachusetts lack proximity to robust charity care due to the lack of major hospitals.

In areas of western Massachusetts, community hospitals dominate the healthcare industry.⁵⁰ Because these community hospitals are in areas of low population density and lack the

⁴⁷ "Health Safety Net," Mass.Gov, <https://www.mass.gov/orgs/health-safety-net>.

⁴⁸ "Is There Financial Help for My Medical Bills?," Consumer Financial Protection Bureau, December 7, 2023, <https://www.consumerfinance.gov/ask-cfpb/is-there-financial-help-for-my-medical-bills-en-2124/#:~:text=More%20information%20about%20charity%20care,state%20Drum%20financial%20assistance%20programs>.

⁴⁹ "Medical Bill Help from Massachusetts General Hospital," Dollar For, <https://dollarfor.org/medical-debt-help-boston-massachusetts-general-hospital/#:~:text=Who%20Qualifies>.

⁵⁰ "Community Hospitals at a Crossroads: Findings from an Examination of the Massachusetts Health Care System," Mass.Gov, March 2016,

resources of federally-funded institutions akin to those in the eastern region of the state, they lack the monetary resources to provide robust charity care. The disparity of charity care across Massachusetts creates an inequitable distribution of medical debt relief programs that varies not necessarily by necessity, but rather by geographic luck.

ConnectorCare

While the Massachusetts Health Safety Net is accessible for residents below 300% of the Federal Poverty Level (FPL), at most, ConnectorCare incorporates a larger spectrum by assisting those up to 500% of the FPL.⁵¹ As another alternative program for MassHealth, ConnectorCare also provides benefits to individuals or families who do not have employment-supported insurance. Receiving federal funding through the Affordable Care Act, the program offers low premiums and copays without deductibles to its users, covering more than 200,000 residents in Massachusetts in 2021.⁵² With programs such as HSN and ConnectorCare, Massachusetts has strengthened its state-run financial inclusivity by providing alternative avenues to access medical care for low-income populations that are ineligible for formal medical insurance.

Low-Income Children with Special Health Conditions in Massachusetts

Narrowing the scope of the topic, medical debt for low-income households with families has been detrimental, especially for children with symptoms that need special medical treatment. These children are often diagnosed with conditions such as genetic and metabolic disorders,

<https://www.mass.gov/doc/community-hospitals-at-a-crossroads-findings-from-an-examination-of-the-massachusetts-health/download>.

⁵¹ “ConnectorCare Health Insurance.” 2025. Mass Legal Help. 2025.

masslegalhelp.org/health-disability-rights/health-insurance-masshealth/connectorcare-health-insurance.

⁵² Howitt, Katherine. 2021. “MassHealth and ConnectorCare Enrollment Tracker.” Blue Cross Blue Shield of Massachusetts Foundation. February 2021.

congenital anomalies, anoxic brain injuries, and cerebral palsy.⁵³ In 2022, 10.9% of Massachusetts families that have children with health needs faced difficulties paying for medical bills, 19.6% of which cut back on their expenses or stopped working as a result. While around half of Massachusetts families with children with health conditions expressed having sufficient insurance for medical expenses, 21.2% of such families indicated having to spend \$1,000 to \$5,000 in out-of-pocket healthcare costs. As such, high costs of pediatric healthcare for children with special medical attention have posed a significant problem for low-income families who are not covered by insurance or other financial sources.

Catastrophic Illness in Children Relief Fund (CICRF)

To mitigate such difficulty, the Massachusetts Department of Public Health established the Catastrophic Illness in Children Relief Fund (CICRF) in 2000 and continues to support families to afford ambulatory care and services from hospitals of pediatric specialty through this initiative. Families with children with medical expenses exceeding 10% of the first \$100,000 of their income and 15% of any income more than \$100,000 are eligible. In 2022 alone, the CICRF supported 160 families with over \$1.98 million⁵⁴ primarily through funding vehicle purchases (transport vehicles and accessible home facilities based on medical conditions). While the children had health insurance, the fund covered insufficient financial assistance, especially with 77% of the families participating in the program having incomes under 300% of the FPL. In contrast to other basic pediatric healthcare service plans in Massachusetts, like the Children's Medical Security Plan,⁵⁵ the CICRF's capabilities in supporting children with medical conditions

⁵³ Goldstein, Robbie. 2025. "The Catastrophic Illness in Children Relief Fund Annual Report Fiscal Year 2023." Commonwealth of Massachusetts.

⁵⁴ Massachusetts Department of Public Health 2022. *Massachusetts Catastrophic Illness in Children Relief Fund Annual Report Fiscal Year 2022*.

⁵⁵ "Children's Medical Security Plan." 2025. Commonwealth of Massachusetts. 2025. mass.gov/childrens-medical-security-plan.

on the severe end of the healthcare spectrum demonstrate Massachusetts's inclusivity for addressing medical debt.

As one of the leading states in the country for pediatric healthcare and insurance,⁵⁶ Massachusetts has continued to establish many financial incentives to expand its medical coverage for children, not only limited to those insured under Massachusetts' Medicaid program, MassHealth. Most recently, the state amended Section 10F of Chapter 118E of the General Laws regarding Children's Medical Security Plan, an alternative medical plan for ineligible children, to remove copays for its members,⁵⁷ making further legislative actions to expand pediatric medical access.

One of Massachusetts' strengths in pediatric healthcare access is its Catastrophic Illness in Children Relief Fund (CICRF), which serves to support low-income families with children having special health conditions by providing payments or reimbursements for health services, such as ambulatory care and transport vehicle purchases. Massachusetts primarily collects funding for this program through Employer Medical Assistance Contribution, which is a form of payroll tax to fund Massachusetts' healthcare initiatives for which employers annually pay contributions on the first \$15,000 of their employees' wages based on a particular rate.⁵⁸ However, audits of the fund have identified several problems and areas of improvement for the program regarding reimbursements and application review periods.

⁵⁶ "Ranking the States on Child Health Care." *Commonwealthfund.org*, 2025, www.commonwealthfund.org/publications/newsletter-article/ranking-states-child-health-care. Accessed 13 Oct. 2025.

⁵⁷ "Section 59: Eliminating Copays for Children's Medical Security Plan | Governor's FY25 Budget Recommendation." *Budget.Mass.gov*, 2025, budget.digital.mass.gov/govbudget/fy25/outside-section/section-59-eliminating-copays-for-childrens-medical-security-plan/. Accessed 13 Oct. 2025.

⁵⁸ "Learn about the Employer Medical Assistance Contribution (EMAC)." *Mass.gov*, 2025, www.mass.gov/info-details/learn-about-the-employer-medical-assistance-contribution-emac. Accessed 13 Oct. 2025.

An audit in 2022 first found that reimbursements for medical expenses totaling around \$24,455 did not include any required documentation to verify the expenses made and determine the appropriate amount of reimbursement.⁵⁹ Although having the requirement of documentation on its eligibility policy, the CICRF did not explicitly go over this process, nor did it use its sliding fee scales as listed under the relevant section (Chp. 111K Section 5) under Massachusetts General Law to determine the amount of reimbursement based on family size and income. Problems such as the absence of documentation, misfiling reports, and unclear calculation of reimbursement can create a lack of transparency and real or perceived misuse of funds that could otherwise potentially be used for those in more financial or medical need. In fact, this led to the program's failure to pursue federal matching funds of around \$49,588 for medical services.

This audit also observed a significant increase in application review time—an increase of 11 days for individuals and an average of 300 total days for families—since that of the previous audit in 2017.⁶⁰ Out of 363 applications, the audit found that 62 applications were idle for approximately 170 days before being matched to a CICRF staff member for review, only to be approved for payment around 140 days after.⁶¹ This delay in time is especially detrimental to families below the federal poverty level and children waiting for urgent support regarding their special health needs, exacerbating the fundamental problem of medical debt in Massachusetts.

These issues in the obscure documentation process, eligibility criteria, and long delays in queues for financial assistance approval seem to all point to what could be a fundamental issue of

⁵⁹ “The Catastrophic Illness in Children Relief Fund Reimbursed Applicants \$24,455 for Medical Expenses without Required Documentation.” *Mass.gov*, 2022, www.mass.gov/info-details/the-catastrophic-illness-in-children-relief-fund-reimbursed-applicants-24455-for-medical-expenses-without-required-documentation. Accessed 13 Oct. 2025.

⁶⁰ “Second Audit in Five Years Finds Longer Delays in Providing Financial Relief for Families of Children with Catastrophic Medical Conditions.” *Mass.gov*, 2017, www.mass.gov/news/second-audit-in-five-years-finds-longer-delays-in-providing-financial-relief-for-families-of-children-with-catastrophic-medical-conditions. Accessed 13 Oct. 2025.

⁶¹ “CICRF Did Not Process Applications in a Timely Manner.” *Mass.gov*, 2018, www.mass.gov/info-details/cicrf-did-not-process-applications-in-a-timely-manner. Accessed 13 Oct. 2025.

the program: staffing shortages. The website of the fund itself explicitly states the potential delay in communication from this issue,⁶² emphasizing the lack of workforce. By replenishing staff or finding alternative ways to cultivate the overall accountability and communicative capabilities between the program and its members, the CICRF could be improved to more efficiently serve its target population and alleviate medical debt for children with special health conditions.

Addressing Massachusetts' Medical Debt Burden

Increasing funding and Incentives for Community Health Centers

In recent years, Massachusetts has increasingly shifted from lower-cost health treatment options, such as community hospitals, community high payer public hospitals, and teaching hospitals, to more expensive care centers like academic medical centers and specialty hospitals. This shift is responsible for some of the overall increases in health care spending and, therefore, medical debt.

A cycle starting with stigma continually pushes individuals away from utilizing community health centers. The belief that community health centers provide worse care directs individuals toward more expensive private and specialty options. Doctors further reinforce the non-community health center bubble by only recommending patients to options outside of community health. As a result, the community health centers end up underfunded, as the main source of funding – commercial payers – continue to choose other options. Community health centers also continue to lose money on unpaid bills. These factors cause community health centers to fare poorly financially with untenable margins, and in some cases, these centers are

⁶² “Catastrophic Illness in Children Relief Fund (CICRF).” *Mass.gov*, 2025, www.mass.gov/catastrophic-illness-in-children-relief-fund-cicrf. Accessed 13 Oct. 2025.

forced to close. Additionally, this lack of funds causes problems in retaining providers, especially because they do not have the bandwidth to pay them as much.

While these centers struggle to effectively function, they can not compete for patients or publicly increase their value to their community, thereby reinforcing the stereotype that they provide subpar care.

To increase the utilization of community health centers, especially by commercial payers, community health centers need not only more funding but also the support of providers in non-community health sectors. Legislation can appropriate more funds for community health centers to allow them to incentivize workers and fill gaps, and also create programs advancing community health centers to address stigma. By increasing the functionality of community health centers and bolstering trust in the community, legislators can tackle the issue of rising health costs and the medical debt associated with that.

Additionally, with the passage of the Big Beautiful Bill, community health centers will become more important than ever as health costs for both Medicare and non-Medicare recipients will skyrocket. It will be imperative to have a functioning, trusted, lower-cost alternative.

Reducing Service Costs

The increase in total health care spending is predominantly due to the increase in service costs. According to the Massachusetts Health Policy Commission's 2024 annual health care costs trends report, outpatient spending and pharmaceutical prices were the primary drivers of increases in health costs. However, costs grew in all areas. The same report found that inpatient spending rose 17%, outpatient spending rose 15.4%, and physicians' office prices rose 12.6%. The rise in costs makes it difficult for all patients to pay, but especially commercial payors. As

prices rise, the margin. By implementing stronger oversight and transparency methods, legislators can address the exorbitant cost increases.

Enforcement of Community Benefits and Financial Assistance by Tax-Exempt Hospitals

The IRS has not used its authority to enforce financial assistance standards in nonprofit hospitals by threatening to revoke their nonprofit status. Thus, though tax-exempt hospitals are meant to use their exemptions towards free and reduced care for low-income populations, tax-exempt and tax-paying hospitals charge similar numbers to low-income individuals.⁶³ Furthermore, the government accountability office's 2020 analysis of IRS data found that there were 30 hospitals that didn't report any spending on community benefits in 2016.⁶⁴

Community benefits can be defined as six various possibilities, including: 1) operating an emergency room available to anyone regardless of if they can pay, 2) having a board of independent civic directors from the community, 3) having an open medical staff policy, 4) providing care for anyone able to pay without discrimination based on factors such as using Medicare or Medicaid, and 5) using funds to improve facilities, equipment, and patient care or 6) to improve training, education and research.² The issue is that there are no requirements for what has to actually be met in the broad range of possible ways of compliance, nor is there any enforcement on compliance, as seen by how some hospitals reported no spending on community benefits.

Massachusetts has a state-managed program called the Health Safety Net that provides financial assistance and also recommends, through voluntary guidelines, that there must be

⁶³ Cooper, Zack, Stuart V. Craig, Martin Gaynor, and John Van Reenen. 2019. "The Price Ain't Right? Hospital Prices and Health Spending on the Privately Insured." *The Quarterly Journal of Economics* 134 (1): 51–107. <https://doi.org/10.1093/qje/qjy020>.

⁶⁴ "United States Government Accountability Office." 2023. <https://www.gao.gov/assets/gao-23-106777.pdf#page=10>.

community benefits for nonprofit hospitals to receive state tax exemptions.⁶⁵ However, there are no minimum requirements for use of funds towards community benefit or charity care beyond the initial licensure requirements.³ The initial license requires nonprofit and for-profit hospitals to provide charity care.³ In addition, Massachusetts does not require community benefit reporting and only has voluntary guidelines for annual reporting.³ Examples of states that do have some minimum requirements are Utah and Illinois, which require that nonprofit tax-exempt hospitals have community benefits equal to the amount that the property tax liability would have been.⁶⁶

A study by the Lown Institute found that from 2020 to 2022, 58% of nonprofit hospitals had a “fair share deficit”, in which they received more tax benefits than they actually spent towards community benefits and health.⁶⁷ The total fair share deficit was 1 billion annually, which is enough to wipe out all medical debt in the state of Massachusetts.⁵ There was also a lot of inconsistency in reported spending by hospitals to the IRS versus to the state in 11 hospitals.^{5 5} Therefore, it seems that greater enforcement of requirements for community benefit and charity care spending is needed through better reporting systems, as well as minimum requirements.

Interventions to Address Unequal Health Access for MA Immigrants

Many immigrants arriving in Massachusetts tend to settle in historically redlined neighborhoods due to lower property values and more affordable housing. However, these neighborhoods often come with limited healthcare infrastructure, higher rates of uninsurance, and greater dependence on public health programs. Additionally, immigrants may have a harder

⁶⁵ “Community Benefit Requirement.” n.d. Accessed October 13, 2025.

https://www.hilltopinstitute.org/wp-content/uploads/hcbp/hcbp_docs/HCBP_CBL_MA.pdf.

⁶⁶ “State Protections against Medical Debt: A Look at Policies across the U.S.” 2023. Commonwealthfund.org, September. <https://doi.org/10.26099/7m6v-ve80>.

⁶⁷ “Making the Hospital Tax Exemption Work for Massachusetts.” n.d. Accessed October 13, 2025. https://lownhospitalsindex.org/wp-content/uploads/2025/04/Massachusetts_state-report.pdf.

time accessing state health insurance, have deep-rooted mistrust in healthcare systems, and in this sociopolitical climate, face fear of deportation.

Current interventions include limited health insurance for emergency care only, “sliding scale” low-cost community health centers that adjust fees based on income, and community care vans that provide free preventive health screenings. However, many, if not all, of these interventions are downstream solutions that act as temporary fixes for a much larger systemic issue. Evidence suggests that historic redlining functions as an upstream barrier to social mobility by limiting individuals' ability to accumulate wealth, which in turn restricts access to quality, comprehensive healthcare. What is needed is upstream reform, which begins with policy changes.

Modifying the Outdated Financial Assistance System

Many hospitals have a designated “sliding scale,” which determines how large a discount patients can receive for services provided, based on the patient’s income in relation to the federal poverty level. For those individuals who are low-income and eligible for some kind of care discount, financial assistance (also known as “Charity Care”) programs provide free or discounted healthcare for specific hospital services and treatments. In states like California and Washington, these Charity Care models ensure low-income individuals who are not covered by the Affordable Care Act (ACA) are able to receive aid. Components of these statutes can be broadly applied to the nonprofit federal tax exemption requirements to expand coverage for low-income individuals and lessen mass medical debt collection.

Currently, hospitals are able to charge at essentially whatever rate they desire for procedures and treatments with little to no oversight; this is known as the so-called

“chargemaster rate.” Often, such prices are arbitrary and extremely high, meaning these chargemaster rates are entirely unrelated to the actual costs of supplies and services.⁶⁸

These prices are devastating for low-income patients, who are unable to negotiate prices with the hospital. In fact, a 2015 study of fifty hospitals found that all charged uninsured patients tenfold the actual cost of the services they received.⁶⁹ For low-income patients, medical debt is often followed by aggressive medical debt collection, which can be followed by financially devastating outcomes like wage garnishing, property liens, and even arrest warrants.⁷⁰ By implementing greater regulation of the financial assistance program through strict reporting guidelines and clear income criteria, low-income patients will face less scrutiny from hospitals and their medical debt collectors.

Implementing Means-Based Deductibles/Pre-Deductible Coverage to Mitigate Medical Debt for Childbearing Individuals

According to a study of 14,560 commercially-insured pregnant people and 12,157 postpartum individuals, it was found that medical debt collections were more likely among those who were postpartum than pregnant. Additionally, those from low-income neighborhoods were disproportionately impacted. Overall, postpartum individuals in lowest income neighborhoods faced the highest probability of being impacted by medical debt collection; pregnant individuals in lowest income neighborhoods were the second most likely. There are legislative means through which to reduce maternal-infant out-of-pocket spending. By implementing means-based deductibles, those from lower incomes would have to pay less for their medical costs.

⁶⁸ Margarita Kutsin, “A Prescription for Charity Care: How National Medical Debt Ills Can Be Alleviated by Integrating State Financial Assistance Policies into the Nonprofit Tax Exemption,” *Seattle University Law Review* 42, no. 965 (2018): 965-992, <https://digitalcommons.law.seattleu.edu/cgi/viewcontent.cgi?article=2593&context=sulr>.

⁶⁹ Kutsin, “A Prescription for Charity Care,” 968.

⁷⁰ Kutsin, “A Prescription for Charity Care,” 970.

Additionally, positing pre-deductible coverage would prohibit out-of-pocket spending for essential peripartum services.⁷¹

⁷¹ Michelle H. Moniz, Molly J. Stout, Giselle E. Kolenic, et al., “Association of Childbirth with Medical Debt,” *Obstetrics and Gynecology* 143, no. 1 (2024): 11-13, <https://pmc.ncbi.nlm.nih.gov/articles/PMC11014426/>.

Key Case Studies

Healthcare Access and Stabilization Program (HASP), North Carolina

HASP is a Medicaid-directed payment program in North Carolina that was established in 2023 in order to improve access to healthcare, to support the state’s broader healthcare reform initiatives, and to expand hospital initiatives providing debt relief to patients.⁷² It is important to note that North Carolina’s system for providing medical debt relief to patients differs from the system in Massachusetts: the Health Safety Net (HSN) in Massachusetts is a state-administered program funded by a state-controlled trust fund—the HSN Trust Fund. HASP, conversely, is a hospital-administered program for debt relief funded by reimbursements from the State of North Carolina. While both HSN and HASP differ drastically in their means of funding, both programs provide mass-debt relief to patients. HASP is an example of a drastic funding increase for debt relief programs similar to that of a \$30 million increase in HSN funding as legislated in Bill H.1354.⁷³ Since the funding increase implemented via HASP in North Carolina is comparable to the extent of funding increase for HSN detailed in Bill H.1354,⁷⁴ the direct effects of HASP implementation on the citizenry of North Carolina may be quite similar to the effects of an increase in HSN funding in Massachusetts.

HASP leverages hospital financing for Medicaid in order to reimburse hospitals with nearly double their contribution via the Federal Medical Assistance Percentage⁷⁵. In order to qualify for HASP reimbursements, qualifying hospitals must implement policies directly tied to

⁷² “What Is the Healthcare Access and Stabilization Program (HASP)?,” *North Carolina Institute of Medicine*, October 9, 2024, <https://nciom.org/what-is-the-healthcare-access-and-stabilization-program-hasp/>.

⁷³ An Act to Address the Financial Sustainability of the Health Safety Net, H.1354, Massachusetts House of Representatives 194, Title XVII, Chapter 118E Massachusetts General Laws, <https://malegislature.gov/Bills/194/H1354/BillHistory>.

⁷⁴ H.1354, Massachusetts House of Representatives 194.

⁷⁵ *Federal Register*, 89, no. 230 (2024): 94742–46.

medical debt relief.⁷⁶ When hospitals implement policies tied to medical debt relief, they receive more funding from the State of North Carolina, allowing hospitals greater ability to absorb uncollected costs of care and provide charity care assistance to patients struggling to pay for medical procedures. HASP has directly contributed to the purchase of \$6.5 billion in medical debt for 2.5 million North Carolina citizens⁷⁷, showcasing the effectiveness of policy initiatives targeted at directly increasing funding for medical debt relief programs.

Community Health and Outreach Program, Fall River, Massachusetts

In Fall River, Massachusetts, about an hour outside of Boston, Saint Anne’s Hospital, a Catholic community hospital in Brown University’s health system, developed a Community Health and Outreach Program focused on providing healthcare while being cognizant of social and economic nuances. As part of the program, the hospital partners with local senior homes, public schools, shelters, and pharmacies to offer regular health education seminars and programs, support groups, and other community outreach initiatives, in hopes of providing as many resources to a diverse population of patients.⁷⁸ In particular, the hospital has been home to the Health Insurance Enrollment and Outreach Program since July 2000, which provides bilingual health promotion advocates who assist both the uninsured and underinsured in understanding all their coverage options and filling out required applications.⁷⁹ In 2023, the Health Insurance

⁷⁶ “What Is the Healthcare Access and Stabilization Program (HASP)?”

⁷⁷ “Governor Stein, NCDHHS Announce More Than \$6.5 Billion in Medical Debt Erased in North Carolina: More than 2.5 Million North Carolinians Relieved of Medical Debt,” *North Carolina Office of the Governor* (North Carolina), October 13, 2025, <https://governor.nc.gov/news/press-releases/2025/10/13/governor-stein-ncdhhs-announce-more-65-billion-medical-debt-erased-north-carolina-0?>

⁷⁸ “Community Health and Outreach at Saint Anne’s Hospital | Brown University Health,” Brown University Health, 2023, <https://www.brownhealth.org/locations/saint-annes-hospital/about-saint-annes-hospital/community-health-and-outreach>.

⁷⁹ “Community Health Partners and Programs at Saint Anne’s Hospital | Brown University Health,” Brown University Health, 2025,

Enrollment and Outreach Program provided programming and insurance assistance to 2,143 patients, with 103 finding immediate coverage.⁸⁰

*Community Service Plans, New York State: New York Public Health Law § 2803-l and § 2807-k(9-a)*⁸¹

Under this statute, all hospitals in New York State are required to file a “Community Service Plan” (CSP) with the New York State Department of Health. Each CSP must include primary service area, community health needs within each area, plans to address those needs, and information about institutional charity care policies and outcomes.⁸² The data collection process for each hospital in New York State is quite similar to that which is proposed in Bill S.857⁸³ for each Massachusetts hospital; however, the New York State statute includes a data collection variable for geographic location. Under § 2807-k(9-a), the primary service area is defined by ZIP codes, counties, or neighborhoods where a majority of patients reside.⁸⁴ This data can be used by the New York State Department of Health to assess geographic disparities in access to healthcare, to identify areas with little access to financial assistance, and, most importantly, to inform the distribution of state-administered medical debt relief.

New York State operates state-administered medical debt relief under a program titled Indigent Care Pool,⁸⁵ similar to that of the HSN in Massachusetts. In New York State, statutes

<https://www.brownhealth.org/locations/saint-annes-hospital/about-saint-annes-hospital/community-health-and-outreach/partners>.

⁸⁰ “2023 SAH Community Health Benefits Programs and Outreach – Results” (Saint Anne’s Hospital, 2023), <https://www.brownhealth.org/sites/default/files/2024-10/2023-SAH-CB-Programs-Results-2024-CB-Exec-Sum-FIN-AL-rev-092424.pdf>.

⁸¹ New York Public Health Law § 2803-l and § 2807-k(9-a).

⁸² New York Public Health Law § 2803-l and § 2807-k(9-a).

⁸³ An Act to Address Medical Debt through Hospital Financial Assistance Reform, S.842, Massachusetts Senate 194, Title XVII, Chapter 118E Massachusetts General Laws, <https://malegislature.gov/Bills/194/S842/BillHistory>.

⁸⁴ New York Public Health Law § 2803-l and § 2807-k(9-a).

⁸⁵ “Examining How Indigent Care Pool Payments Are Allocated in New York,” *Research Foundation of the City University of New York*, December 2017,

exist to ensure proper collection of data that can better inform statewide medical debt relief programs.⁸⁶ However, there are currently no state statutes in Massachusetts that detail precisely how data should be collected for patients who apply for medical financial aid. Bill S.857 aims to address this very issue by standardizing data collection processes on patients who apply for financial aid for hospitals in Massachusetts.⁸⁷ However, Bill S.857 misses a key component of data collection: geographic location. The success of New York Public Health Law § 2803-l and § 2807-k(9-a) in informing statewide decisions on state-funded medical debt relief programs highlights the value of standardized data collection processes, which include geographic data, allowing state programs to formulate strategies for addressing rampant place-based inequality.

Centers for Medicare and Medicaid Services Hospital Price Transparency Rule

The Centers for Medicare and Medicaid Services, also known as the CMS, enacted a Hospital Price Transparency Rule—effective since January 1, 2021—mandating that all US hospitals, including both nonprofit and for-profit institutions, provide clear and readily available pricing information online for every single item or service that they were or will be offering. Hospitals are also required to provide this data in both a machine-readable comprehensive file, along with a consumer-friendly format for shoppable services like childbirth, joint replacements, or mammograms. This policy is one of the largest federal interventions to make hospital prices more transparent in the US. CMS undertakes very frequent audits of hospitals to ensure compliance and is empowered to issue civil monetary penalties to those who are found non-compliant. They could be \$300 to \$5500 per day, depending on the hospital size, and there

[https://nyhealthfoundation.org/grant-outcome/examining-how-indigent-care-pool-payments-are-allocated-in-new-york/#:~:text=The%20indigent%20care%20pool%20\(ICP\)%20is%20a,to%20uninsured%20patients%20*%20Insufficient%20Medicaid%20rates](https://nyhealthfoundation.org/grant-outcome/examining-how-indigent-care-pool-payments-are-allocated-in-new-york/#:~:text=The%20indigent%20care%20pool%20(ICP)%20is%20a,to%20uninsured%20patients%20*%20Insufficient%20Medicaid%20rates).

⁸⁶ New York Public Health Law § 2803-l and § 2807-k(9-a)

⁸⁷ S.842, Massachusetts Senate 194

have been a noticeable number of high-profile hospitals found for incomplete or even inaccessible pricing data.

Maryland Hospital Financial Assistance and Demographic Reporting

Maryland has enacted significant hospital financial assistance and reporting regulations; since 2017, Maryland hospitals have been required to publicly disclose the total amount of financial assistance granted, the amounts written off as bad debt, and the number of patient accounts sent to collections agencies, which can result in lawsuits or wage garnishments. In 2023, the reforms extended the transparency mandate, now requiring hospitals to break these numbers up by race, ethnicity, gender, zip code, language, etc.—anything that can indicate demographics—in order to find gaps in assistance and help enable more targeted interventions. The Maryland law also provides protections during financial aid application and approval timelines. For instance, hospitals have to demonstrate that patients received adequate notice of available assistance programs before sending bills to collections or proceeding with legal action. Maryland’s Health Services Cost Review Commission also annually publishes reports identifying hospitals failing to comply with these measures, facing regulatory sanctions, more oversight, or reduced reimbursement.

Spartanburg Regional Healthcare Interpreter on Wheels, Spartanburg, South Carolina

In Spartanburg, South Carolina, hospitals have seen a steady increase in non-English speaking patients, with more than 6% of those in the region not speaking any English, and 4% of residents being immigrants. As a result, the Spartanburg Regional Healthcare system sought to effectively and efficiently break down such language barriers by utilizing Interpreter on Wheels

(IOW).⁸⁸ As desirable in-person interpreters would be, when hospitals are already struggling with scheduling interpreters, unexpected delays, and consistent shortages, implementing virtual interpreters is a temporary solution.⁸⁹ IOWs can be a cost-effective solution, as face-to-face interpreters often have a minimum of two-hours and can range anywhere from \$45 to \$150 per hour, on average.⁹⁰ On the other hand, virtual interpreters offer much more freedom with time, often costing \$1.95 to \$3.49 per minute.⁹¹ With the implementation of Interpreter on Wheels, the Spartanburg Regional Healthcare system has now been able to provide genuine access to medical interpretation 24 hours a day, seven days a week.⁹²

Hospital Financial Assistance and Demographic Reporting. Maryland: Enacted in 2017.

Updated and Expanded in 2023

Maryland enacted hospital financial assistance and reporting regulations; since 2017, Maryland hospitals have been required to publicly disclose the total amount of financial assistance granted, the amounts written off as bad debt, and the number of patient accounts sent to collections, lawsuits, or wage garnishments. In 2023, the reforms extended the transparency mandate, now requiring hospitals to break these numbers up by race, ethnicity, gender, zip code, language, etc.—anything that can indicate demographics—in order to find gaps in assistance and help enable more targeted interventions.

⁸⁸ Julie Carson, “CASE STUDY: South Carolina Hospital Uses Video Remote Interpreting,” *LanguageLine.com*, 2019, <https://www.language-line.com/blog/how-regional-hospitals-in-the-south-are-using-video-remote-interpreting-to-communicate-with-multicultural-patients?>

⁸⁹ Fady Tawfik, “Why Hospitals Struggle with Interpreter Scheduling (and How to Fix It),” *Ad-astrainc.com* (Ad Astra, Inc., May 21, 2025), <https://ad-astrainc.com/blog/why-hospitals-struggle-with-interpreter-scheduling-and-how-to-fix-it>.

⁹⁰ “How to Pay for Translation Services,” *Apa.org* (American Psychological Association, 2024), <https://www.apa.org/monitor/2020/01/career-other-words-sidebar>.

⁹¹ *Apa.org*, “How to Pay for Translation Services.”

⁹² “Language Services,” *www.spartanburgregional.com*, n.d., <https://www.spartanburgregional.com/patients-visitors/language-services>.

The Maryland law also provides protections during the application and approval process. For instance, hospitals have to demonstrate that patients received adequate notice of available assistance programs before sending bills to collections or going forth with legal action. Maryland's Health Services Cost Review Commission also annually publishes reports identifying hospitals failing to comply with these measures, facing regulatory sanctions, more oversight, or reduced reimbursement.

All Maryland hospitals have to report the numbers of financial assistance applications received, approved, denied, and appealed to the state. Hospitals have to disclose the number and demographics of accounts tied to extraordinary collection actions. The Annual reports have to break down race, ethnicity, and gender. As of 2024, hospitals approved over 1.2 billion dollars in financial assistance annually, and the data was filtered depending on race and zipcode. Expanded transparency resulted in 17% increase in financial assistance applications received, and 12% reduction in hospital lawsuits against patients from disadvantaged zip codes post-reform implementation.⁹³

Promoting Increased Access to Free Health Care for Indigent Persons, Illinois: Illinois SB 2194, 2012

In June 2012, Senate Bill 2194 was signed into law. The intent of this bill was to “promote increased access to free health care for indigent persons and strengthen the medical assistance program” by establishing “quantifiable standards” for hospitals to receive and maintain tax exempt status.⁹⁴ Accordingly, in order to obtain and maintain tax exemption, a

⁹³ “Hospital Price Transparency Posting Evaluation Report.” n.d.

<https://hcpf.colorado.gov/sites/hcpf/files/Price%20Transparency%20Postings%20Evaluation%20Report.pdf>.

⁹⁴ “Illinois Governor Patrick Quinn Signs Tax Exemption and Tax Credit Legislation Affecting Hospitals.” 2025. Hinshaw & Culbertson LLP. Hinshaw & Culbertson LLP. 2025.

<https://www.hinshawlaw.com/en/insights/healthcare-alert/illinois-governor-patrick-quinn-signs-tax-exemption-and-tax-credit-legislation-affecting-hospitals>.

hospital needs to prove that the value of services or activities for a hospital year is at least equal to the hospital's estimated property tax liability for that year through an application.⁹⁵ They may either use the value of activities/services for the hospital fiscal year or an average value of activities/services for 3 fiscal years ending with the hospital year; the latter choice accounts for unusual one-time events and year to year fluctuations.¹³ Some specifications are also made: for multi-hospital systems, the value of the services and activities and the property tax liability will be calculated only according to each individual hospital.¹³ Thus, a hospital system cannot pool charity care services for one hospital as justification for tax exemption for another hospital in that system. For a hospital system that spans across multiple states, the value of services and activities and the property tax liability will only be calculated with respect to the services/activities and property located in Illinois.¹³

Additionally, any portion of a hospital that is owned by a for-profit entity or even simply leased, licensed, or operated by a for-profit entity cannot qualify for tax exemption.¹³ Lastly, if a portion of a hospital has some tax-exempt and non-exempt uses, only the qualifying portion will be exempt, and for shared spaces that serve both tax-exempt and non-exempt uses, they can receive partial exemptions that are proportional to the amount of tax-exempt purpose.¹³ For example, if floor 1 is nonprofit hospital services, it will be exempt, but if floor 2 is for a for-profit group, it will not be exempt. Similarly, a parking garage that's 50% used by hospital staff and patients and 50% used for for-profit people will be proportionally 50% tax exempt. These specifications prevent hospitals from taking advantage of tax-exempt status and ensure that charitable use is *directly* tied to tax-exempt status.

⁹⁵ "35 ILCS 200/15-86." 2025. Ilga.gov. 2025.
<https://www.ilga.gov/Documents/legislation/ilcs/documents/003502000K15-86.htm>.

Furthermore, the services and activities that qualify to be included in the report include the following: 1) Charity care through free and discounted services. 2) Health services for low-income and underserved individuals. 3) Direct or indirect subsidies to state or local governments for health-related activities/programs for low-income and underserved individuals. 4) Support for state health care programs for low income individuals, calculated either by counting 10% of payments towards means-tested (i.e. based on income) public health programs such as Medicaid or by calculating the actual unreimbursed costs of these programs, through the same method it is calculated on IRS Form 990, Schedule H.¹³ They can choose between the two options each year they apply.¹³ 5) Dual eligible (for Medicaid-Medicare) subsidies can also be counted, calculated by multiplying total Medicare unreimbursed costs to the ratio of dual eligible patients to total Medicare patients.¹³ Since Medicare is not means-tested (i.e. based on income) while Medicaid is, this ensures that the portion of Medicare expenses attributable to dual eligible patients can be counted as services towards low income patients. 6) Any unreimbursed costs of essential activities or services that relieve the burden of the government for healthcare of low-income individuals, such as emergency care or medical education, calculated through standardized methods.¹³ 7) Any other activities that the Department acknowledges as relieving the burden of the government or serving low-income or underserved individuals.¹³ By specifying what qualifies as a service/activity, Illinois decreases the ambiguity of “community benefit” and quantifies standardized ways to calculate each type of service/activity.

The Illinois Attorney General’s Office reviews nonprofit hospital community-benefit reports and ensures the hospitals’ charitable activities justify their tax-exempt status each year.⁹⁶ According to the Community Benefits Act, these annual reports sent to the Illinois Attorney

⁹⁶ Legislative Information System. 2024. “Official Government Website of the Illinois General Assembly.” Ilga.gov. June 14, 2024. <https://www.ilga.gov/Legislation/ILCS/Articles?ActID=2465&ChapterID=21>.

General must include a mission statement, community benefit plan, community benefits provided, charity policy, and audited financial statements, and must be made public on the hospital's website.¹⁴ However, public and rural hospitals outside a Metropolitan Statistical Area and small hospitals with less than 100 beds are exempt from filing annually.¹⁴ The state administrators and courts have upheld this framework, such as in the *Oswald v. Hamer* case.⁹⁷

Looking at the impacts of these regulations, a study by the Lown Institute found that despite Illinois being a leading state in policies around community benefit reporting, it still has a relatively high fair share deficit from 2020-2022, of around 1.3 billion.⁹⁸ Possible limitations include a lack of a central location for the reports and property tax information to be collected and publicly available, reducing transparency.¹⁶ There is also no clear enforcement following reporting. It's also relevant to consider that the study likely used a calculation method for finding the fair share spending that differs from the methods outlined in state policy. Still, this data outlines limitations of having a primary focus only on reporting rather than enforcement of minimum requirements, which will be further addressed in the following case study.

A study that provides convincing evidence of the overall impact of community benefit state regulation policies is a national study conducted in 2009-2015, finding that “exposure to a state-level community benefit law of any type was associated with an \$8.42 (95% confidence interval: 1.20-15.64) per \$1000 of total operating expense greater community benefit spending.”⁹⁹ In this case, a community benefit law includes either reporting or minimum threshold requirements.

⁹⁷ “Illinois High Court Finds Statutory Charitable Use Property Tax Exemption Constitutional.” 2018. Ey.com. 2018. <https://taxnews.ey.com/news/2018-1929-illinois-high-court-finds-statutory-charitable-use-property-tax-exemption-constitutional>.

⁹⁸ “Making the Hospital Tax Exemption Work for Illinois.” n.d. Accessed December 22, 2025. <https://lownhospitalsindex.org/wp-content/uploads/2025/04/illinois-state-report-20250416.pdf>.

⁹⁹ Johnson, Emily K, Rose Hardy, Tatiane Santos, Jonathon P Leider, Richard C Lindrooth, and Gregory J Tung. 2019. “State Laws and Nonprofit Hospital Community Benefit Spending.” *Journal of Public Health Management and Practice* 25 (4): E9–17. <https://doi.org/10.2307/48517401>.

Therefore, there is currently no definitive evidence showing that state charity-care minimum and reporting laws alone automatically increase overall community benefit spending or eliminate fair-share deficits — evaluations are limited or mixed. Yet, research suggests that state policies on community benefits do overall have some positive impact towards increasing accountability that deserves further study.

Catastrophic Illness in Children Relief Fund, New Jersey

Founded earlier in 1988 than that of Massachusetts (2000), the New Jersey CICRF financially assists families with children whose health conditions demand high uncovered medical expenses without state or federal insurance. Similar to Massachusetts, New Jersey families are eligible to apply to the CICRF when their medical expenses for children are greater than 10% of the family's income in that year, adding 15% of any income over \$100,000,¹⁰⁰ helping to address medical debt for pediatric healthcare.

Despite being the same program, the CICRF demonstrates a number of differences between the two states. In 2022 alone, New Jersey awarded 156 families totaling around \$3.8 million,¹⁰¹ which is around double the amount of funding provided for each family by Massachusetts. This greater financial balance in New Jersey's program most likely comes from the fact that New Jersey collects "\$1.50 per employee levied on all employers who are subject to the New Jersey Unemployment Compensation Law." This payroll tax is \$0.50 greater than how Massachusetts collects \$1.00 from each employee under employer medical assistance contribution for the CICRF,¹⁰² possibly explaining the difference between the two programs'

¹⁰⁰ "Catastrophic Illness in Children Relief Fund." Catastrophic Illness in Children Relief Fund, 2025, www.nj.gov/humanservices/cicrf/about/about-cicrf.

¹⁰¹ *New Jersey Catastrophic Illness in Children Relief Fund State Annual Report Fiscal Year 2022*.

¹⁰² "Catastrophic Illness in Children Relief Fund (CICRF)." Mass.gov, 2025, www.mass.gov/catastrophic-illness-in-children-relief-fund-cicrf.

financial scope. Furthermore, New Jersey’s fund model is explicitly specified as a “dedicated, revolving, non-lapsing trust fund,” which emphasizes the circular, non-lapsing nature of funding that could reinforce the larger scale of funds in New Jersey CICRF. To expand the financial capabilities of the Massachusetts CICRF, raising employee contributions and establishing a self-reinforcing fund model like that of New Jersey can be solutions to increase fund balance.

One significant difference between the two programs pertains to their administrative structure. Although associated with the Department of Human Services, New Jersey’s CICRF is largely independent of the department’s supervision or control.¹⁰³ This contrasts with the program in Massachusetts, which is legally integrated under the Department of Public Health. While both programs have public representatives from state departments, New Jersey’s administrative independence is pronounced when looking at office staff role descriptions — New Jersey’s state office staff details more specialized positions to its members, such as a fiscal manager, provider relations specialist, and accounting assistant.

Hence, New Jersey CICRF’s separated structure from the state department can possibly explain its greater autonomous efficiency and workforce availability for processing fund application reviews. Compared to New Jersey, the Massachusetts CICRF’s connection in the workforce to state administration perhaps contributes to procedural delays and prevents flexibility in the staff for running the program, exacerbating its ongoing problems of extensive application delays and communication barriers.

¹⁰³ *New Jersey Catastrophic Illness in Children Relief Fund State Annual Report Fiscal Year 2022*. 2022.

Collaboration with Undue Medical Debt, Delaware

Delaware's efforts to alleviate general medical debt are noteworthy and exemplary for improving other state-level medical debt programs like the Massachusetts CICRF. In July 2025, the State of Delaware officially partnered with a national non-profit organization, Undue Medical Debt, to leverage \$500,000 in its state funds to purchase and remove up to \$50 million in medical debt for more than 17,000 Delawareans.¹⁰⁴ Furthermore, most recently in October 2025, a new Delaware law took effect to no longer include medical debt in consumer credit reports, safeguarding families from long-term medical burden.¹⁰⁵

As an independent non-profit organization founded in 2014, Undue Medical Debt purchases bundled medical debt portfolios in large scale from more than 300 partnered donors, such as hospitals and commercial debt buyers. As a result, the organization can help pay medical bills — and has relieved \$23 billion in debt for over 14.72 million people in the US so far.¹⁰⁶

The most effective aspect of Delaware's partnership is that, with Undue Medical Debt, applying for medical reimbursements is no longer a necessity. Although not based on individual requests, the relief system makes it possible for qualifying residents in Delaware and partner states to automatically receive relief through the organization's purchases of debt portfolios. This removes people's need to apply for any medical aid programs, as they are simply notified with a letter informing them that their medical debt has been relieved without any attached taxes,

¹⁰⁴ "Governor Meyer Announces Partnership for \$50 Million in Medical Debt Elimination - State of Delaware News." 2025. State of Delaware News. July 29, 2025. <https://news.delaware.gov/2025/07/29/governor-meyer-announce-partnership-for-50-million-in-medical-debt-elimination/>.

¹⁰⁵ "New Law Excluding Medical Debt from Credit Reporting Takes Effect - State of Delaware News." 2025. State of Delaware News. October 27, 2025. <https://news.delaware.gov/2025/10/27/new-law-excluding-medical-debt-from-credit-reporting-takes-effect/>.

¹⁰⁶ "Undue Medical Debt — Donor-Powered Medical Debt Relief, Liberating Everyday People." Undue Medical Debt. <https://unduemedicaldebt.org/>.

penalties, or strings. This relief process happens every other week, and donations are processed in 2-3 weeks.

Minimum Charity Care Requirements for Hospitals, Nevada

Nevada requires both nonprofit and for-profit hospitals that have at least 100 beds and are located in a county with at least 2 licensed hospitals to have a minimum level of charity care of at least .6 percent of the hospital's net revenue for the preceding fiscal year.¹⁰⁷ Once the minimum amount has been reached, the county may reimburse the hospital for the care of indigent patients.¹⁸ The minimum level is calculated by the Department of Health and Human Services, and the board of county commissioners for each county maintains a record of discharge forms and minimum obligations for the hospitals in their county.¹⁸ Before September 30 of each year, the hospitals must submit a report that includes information such as the total number of indigent patients treated, patients treated with no reimbursement from the county, the total amount paid to each hospital for treatment of indigent patients, and the amount the hospital should have received for unreimbursed patients.¹⁸ If they do not meet these minimum obligations and complete the required reports, the shortfall must be paid to the county (Nev. Rev. Stat. §439B.320 and §439B.340(2)(c)).¹⁸

The Director of the Department of Health and Human Services is responsible for determining how much the hospital must pay.¹⁸ If they fail to pay on time, interest will also accumulate at 1% per month, and the collection can be pursued by the Director through courts; the hospital will have to pay resulting court costs and fees as well.¹⁸ Through these financial consequences, Nevada ensures that there is no way for hospitals to avoid the need to provide the

¹⁰⁷ "NRS: CHAPTER 439B - RESTRAINING COSTS OF HEALTH CARE." 2024. State.nv.us. 2024. <https://www.leg.state.nv.us/NRS/NRS-439B.html>.

required care for indigent populations and also provides a strong incentive towards completing the reporting. Furthermore, nonprofit and for-profit hospitals with at least 200 beds must discount by at least 30% the total charges billed for inpatient medical services to patients who are uninsured and ineligible for publicly funded health coverage.¹⁸ Then they must make reasonable arrangements within the 30 days after a hospital sends the bill for the patients to be notified of available discounts. (Nev. Rev. Stat. §§439B.260 (1) –(2).)

Policy Recommendations

Amend and Enact Existing Bills

Amend and Enact Massachusetts Bill H.1354, An Act to Address the Financial Sustainability of the Health Safety Net

The funding practices laid out in H.1354 would ensure that Health Safety Net (HSN) remains funded despite the increasing cost of healthcare in Massachusetts. Rising (and poorly-regulated) healthcare costs are a result of non-stop booming inflation.¹⁰⁸ As long as inflation patterns continue to follow recent trends, healthcare will become increasingly inaccessible to low-income families. This, in turn, will cause more individuals to rely on debt-relief programs such as HSN. Because of the rising cost of healthcare, the number of HSN beneficiaries will increase; however, to account for the projected increase in beneficiaries, more money must be allocated to HSN in order to aid these new beneficiaries. H.1354 provides a stable funding mechanism to ensure the continuity of HSN.

Background

While government programs aimed at offering assistance to those falling below the poverty line are abundant in Massachusetts—HSN and Massachusetts Charity Care being the two most widely used—these programs are quickly headed toward record-level budget shortfalls.^{109, 110} Despite the dwindling budget of debt relief programs, the percentage of individuals who now rely on HSN to field medical debt who live among the lowest income level

¹⁰⁸ An Act to Address the Financial Sustainability of the Health Safety Net, H.1354, Massachusetts House of Representatives 194, Title XVII, Chapter 118E Massachusetts General Laws, <https://malegislature.gov/Bills/194/H1354/BillHistory>.

¹⁰⁹ John LoDico, “Long-Term Care Report; New Safety Net Numbers,” Massachusetts Health & Hospital Association, August 11, 2025, <https://www.mhalink.org/mondayreport/long-term-care-report-new-safety-net-numbers/#1>.

¹¹⁰ “Massachusetts’ Charity Care Fund Is Running on Empty,” Healthcare for All, March 6, 2025, <https://hcfama.org/massachusetts-charity-care-fund-is-running-on-empty-boston-globe-march-6-2025/>.

has risen by 20 percent,¹¹¹ highlighting a key issue facing Massachusetts residents: an increasing number of individuals suffering from medical debt are becoming more impoverished, but the social programs aimed at mitigating poverty resulting from medical debt are running out of essential funding to maintain their effectiveness.

Bill H.1354 aims to increase state funding for HSN from \$30 million annually to \$60 million annually.¹¹² Additionally, the bill aims to create a payment system in which private insurers cover exactly half of any funding shortfall in the HSN as assessed by the Executive Office of Health and Human Services.¹¹³

Cost-Benefit Analysis

1. **Fund increase cost:** \$30,000,000
2. **Administrative cost:** Between $\$80,186 * 3 = \$240,558$ and $80,186 * 5 = \$400,930$
3. **Total estimated cost per year and recommended appropriation:** \$30,400,930

The typical salary of an administrative manager is \$80,186¹¹⁴ at the Massachusetts Department of Public Health. In order to oversee and manage payments from private insurers, three to five additional administrative managers must be hired. While taxpayers will modestly bear the price of implementing Bill H.1354, the overwhelming majority of the cost of implementing this bill will fall on private insurance corporations. Without increased funding, the HSN shortfall will reach \$250 million by the end of the 2025 fiscal year and potentially \$290 million by the end of the 2026 fiscal year.¹¹⁵ In turn, private health insurance corporations will

¹¹¹ Jeffrey Shapiro, “2025 Annual Report on MassHealth and the Health Safety Net: An Analysis of Health Safety Net Parents, Providers, and Services,” Office of the Inspector General of Massachusetts, March 3, 2025, <https://www.mass.gov/news/oig-releases-annual-report-on-public-healthcare>.

¹¹² H.1354, Massachusetts House of Representatives 194.

¹¹³ H.1354, Massachusetts House of Representatives 194.

¹¹⁴ “Statewide Payroll Open Payroll,” Massachusetts Department of Public Health, accessed November 18, 2025, https://cthrupayroll.mass.gov/#/year/2022/full_time_employees.others/pay1/explore/0-0-0/position_title/Administrator+IV/0/departments_division.

¹¹⁵ John LoDico, “Uninsured Rising. Safety Net Fund Falling,” Massachusetts Health & Hospital Association, June 9, 2025,

likely increase the cost of coverage, raising deductibles and/or premiums. Instead of the taxpayers falling victim to the immense cost of managing debt relief, private insurers and big business will field a greater share of the cost to protect their own employees from the rising cost of healthcare. Increasing funding toward HSN to pay community healthcare providers that participate in MassHealth could potentially increase MassHealth coverage by thousands of individuals, as more hospitals will be inclined to provide care at more reasonable rates.

Limitations and Implementation

The recommendation previously presented is not yet allowed by Massachusetts statute, for the current appropriated funding for HSN was codified in 2020. While increasing government funding for HSN might not pose a major issue, private insurance corporations will most likely be in staunch opposition to this policy recommendation, for they will be partially responsible for covering funding shortfalls. It is expected that private insurance corporations will spend millions of dollars in lobbying against this policy recommendation. To combat the foreseeable aggressive spending by major corporations, grassroots organizations such as Undue Medical Debt and Healthcare for All MA have long intervened in the never-ending medical debt crisis, and their experience in informing public opinion on the structural challenges to tackling medical debt would doubtlessly be useful. Healthcare for All MA spends fundraised money on grassroots lobbying and organizing to affect public policy related to medical debt, while Undue Medical Debt instead appropriates resources to outright abolish medical debt for patients and inform the public on structural issues within the US healthcare system. Grassroots organizing is the best way to combat insurance giants and their foreseeable attempt to outspend Americans into debt crises.

<https://www.mhalink.org/mondayreport/uninsured-rising-safety-net-fund-falling/#:~:text=MHA%20last%20week%20sent%20a,million%20in%20fiscal%20year%202026.>

Amend and Enact Massachusetts Bill S.842, An Act to Address Medical Debt Through Hospital Financial Assistance Reform

Currently, S.842 sets incoming eligibility tiers for patients depending on a given percentage of the Federal Poverty Level (FPL) they are categorized as. However, while the status quo mandates posting notices regarding their financial assistance policy in various languages, it does not require hospitals to automatically screen patients by default. Thus, it is recommended that there be automatic screening in hospitals serving high-need neighborhoods to proactively assist patients before billing or sending them to collections. The City of Boston Planning Department has historically defined high-need neighborhoods with a “Neighborhood Distress Index”, which utilizes factors such as educational attainment, youth violence, and poverty incidence. For example, Roxbury, Dorchester, and Mattapan have the highest index scores in Boston, reaching 70-88. As such, we recommend also utilizing an index of a similar nature and standard across Massachusetts. In order to further ensure linkage to care, hospitals in high-need areas ought to require Community Health Navigators (CHNs) to better assist with language barriers, MassHealth enrollment, financial-assistance forms, and follow-up debt-collection counseling.

Background

The neighborhoods of Boston and regions of Massachusetts that would benefit the most from a Fall River-based framework: Roxbury, Dorchester, and Mattapan, and South Coast,¹¹⁶ respectively, are the same communities that underwent redlining in the late 1960s.¹¹⁷ At the time,

¹¹⁶ “Mapping Inequality,” Richmond.edu, 2025, https://dsl.richmond.edu/panorama/redlining/map/MA/Boston/area_descriptions/D12#mapview=full&loc=12/42.2843/-71.0465.

¹¹⁷ “Redlining,” @Fedhistory, 2023, <http://federalreservehistory.org/essays/redlining>.

this meant controlling the viability and feasibility of which residents received loans. Decades later, historic redlining has manifested into health inequities and lingering institutional disadvantages.¹¹⁸ It connects to why residents of Roxbury and Mattapan are nearly two times as likely to be uninsured compared to the rest of Boston; and why half their residents have “subprime” credit scores (580–619, compared to a “prime” credit score of 660–719), while only 8 percent of Newton and Beacon Hill residents have such scores.¹¹⁹ This is the sort of historical nuance that ought to be taken into account when developing healthcare systems—programs akin to Fall Rivers’ do just that.

Cost-Benefit Analysis

One possible idea for the aforementioned policy recommendation is implementing a standardized automatic-screening protocol and allocating dedicated Community Health Navigators (CHNs) in hospitals that serve high-need neighborhoods. The following cost-benefit analysis draws on national data regarding the cost of hospital financial counselors and CHNs.

1. Cost: Employee Wages

a. Hospital financial counselors

- i. Such financial counselors are expected to “identify and assist patients who are unable to pay . . . or to assist patients who have large existing balances”, and thus are equipped to act as screeners to help relieve financial stress before treatment begins.¹²⁰

¹¹⁸ Leonard E. Egede et al., “Modern Day Consequences of Historic Redlining: Finding a Path Forward,” *Journal of General Internal Medicine* 38, no. 6 (February 6, 2023): 1534–37, <https://doi.org/10.1007/s11606-023-08051-4>.

¹¹⁹ Reserve Bank, “Federal Reserve Bank of Boston,” Federal Reserve Bank of Boston, June 27, 2018, <https://www.bostonfed.org/publications/community-development-issue-briefs/2018/the-concentration-of-financial-disadvantage.aspx>.

¹²⁰ “Financial Counseling,” stanfordhealthcare.org, n.d., <https://stanfordhealthcare.org/for-patients-visitors/financial-counseling.html>.

ii. **Median Annual Salary:** \$57,000¹²¹

iii. Assuming 52 weeks and 40 hours of work per week, **Hourly Wage:**
\$27.40

b. Community Health Navigators (CHNs)

i. **Median Annual Salary:** \$67,000¹²²

ii. Assuming 52 weeks and 40 hours of work per week, **Hourly Wage:**
\$32.21

2. Benefits: More Individualized Guidance and Care for High-Need Communities, also saving money for hospitals themselves.

a. Hospitals lose out on revenue when patients aren't fully informed and equipped to deal with insurance.

i. AnnexMed, Medical billing and Revenue Cycle Management Company, found that hospitals themselves lose 3% to 5% on patient revenue due to lost or denied claims, a consequence of unclear coverage, lack of proper follow-up, etc.¹²³

b. Community Health Workers (CHWs) are far more efficient, decreasing the cost per patient. In a study conducted by the American Hospital Association,¹²⁴ the following benefits from CHWs were reported:

¹²¹ Glassdoor, "Patient Financial Counselor Salaries," accessed November 29, 2025, https://www.glassdoor.com/Salaries/massachusetts-patient-financial-counselor-salary-SRCH_IL.0,13_IS3399_KO14,41.htm.

¹²² Glassdoor, "Community Health Navigator Salaries," accessed November 29, 2025, https://www.glassdoor.com/Salaries/massachusetts-community-health-navigator-salary-SRCH_IL.0,13_IS3399_KO14,40.htm.

¹²³ AnnexMed, "Write-Offs in Medical Billing Is Costing You More than You Think," AnnexMed, March 24, 2025, <https://annexmed.com/understanding-write-offs-in-medical-billing?>.

¹²⁴ American Hospital Association, "Building a Community Health Worker Program the Key to Better Care, Better Outcomes, & Lower Costs," July 2018, <https://www.aha.org/system/files/2018-10/chw-program-manual-2018-toolkit-final.pdf?>.

- i. **Average savings from implementing CHW program:** \$2,245 per patient
 - 1. \$2.28 for every \$1.00 invested into CHW programs
- ii. **Decrease in hospital readmission:** 11.9% decrease in frequency of hospital readmission with patients that had CHW intervention.¹²⁵

Limitations and Implementation

For one, seeing as this would be a mere amendment to current legislation, the current Massachusetts statute ought to have no problem seeing this through. Since hospitals are already expected to have some sort of financial assistance infrastructure, requesting automatic screening and proactive counseling poses no conflict.

Naturally, any legislation requiring a large allocation of funds will have monetary limitations, but ultimately, the return on investment, as mentioned above (\$2.28 for every \$1.00 invested into CHW programs), allows this amendment to fund itself in the long run.

A legislator to reach out to early on would be Senator Joanne M. Comerford, Massachusetts state senator for Hampshire, Franklin, and Worcester, who filed Bill S.842 originally. Senator Comerford has been a long-time supporter of expanding medical loan repayments, creating stronger community-run health centers, and ensuring better protections for patients and physicians alike, and thus would be a valuable resource. Senator Comerford has dedicated extensive amounts of time towards field work, working with local non-profits, visiting community centers, hosting town halls, and more, and thus has a robust understanding of what Massachusetts constituents truly desire, making her an even more valuable resource to ensure effective policies are being enacted.¹²⁶

¹²⁵ Noah Brown, "Mass General Release: A Community Health Worker Intervention Reduces Hospital Readmissions," Massachusetts General Hospital, n.d., <https://www.massgeneral.org/news/press-release/mass-general-release-a-community-health-worker-intervention-reduces-hospital-readmissions>.

¹²⁶ Senatorjocomerford.org, 2025, <https://senatorjocomerford.org/>.

Furthermore, as this policy recommendation is especially community-oriented, it would be appropriate to engage as many community members and organizations as possible. Health Care for All Massachusetts (HCFAMA)¹²⁷ and Community Catalyst¹²⁸ both advocate for health equity and medical debt reform in Massachusetts. Boston CHW Workforce Coalition,¹²⁹ in particular, “was founded by and for CHWs to establish the workforce as a profession”, and would thus be valuable in expanding CHWs across Massachusetts.

Amend and Enact Massachusetts Bill H.101, An Act Establishing Free Broadband Internet Access in Public Housing

Amending Bill H.101 to require not only free broadband access in public housing but also guaranteed broadband infrastructure and internet/AI training for healthcare professionals in rural counties would significantly strengthen Massachusetts’s efforts to close the digital divide and improve health outcomes. While the current bill focuses on providing high-speed internet to vulnerable residents, expanding its scope to include rural healthcare facilities would ensure that both patients and providers benefit from reliable connectivity.¹³⁰ By mandating that all rural hospitals and clinics have access to high-speed broadband, the state would enable telemedicine, digital health records, and online financial assistance programs to function effectively in areas that have historically lagged behind urban centers. This would help reduce medical debt by making it easier for rural patients to access care, understand billing, and apply for financial support online.¹³¹

¹²⁷ “Health Care for All – Advocates for Health,” n.d., <https://hcfama.org/>.

¹²⁸ “Home Page,” Community Catalyst, n.d., <https://communitycatalyst.org/>.

¹²⁹ “About - MACHW,” MACHW, December 2, 2024, <https://machw.org/about/>.

¹³⁰ Sena, Danilo A. n.d. “An Act establishing free broadband internet access in public housing.” MA General Court. <https://malegislature.gov/Bills/194/H101>.

¹³¹ Frobom, Kiersten. n.d. “Broadband initiatives for unserved and underserved areas.” County Health Rankings. Accessed November 11, 2025.

In addition, requiring internet and AI training for healthcare professionals in these rural counties would empower providers to use digital tools and artificial intelligence to improve patient care and administrative efficiency. Training programs could cover telehealth best practices, digital health literacy, and the use of AI for patient management, billing, and outreach. This would not only enhance the quality of care but also ensure that providers can assist patients in navigating online systems, reducing errors and missed opportunities for financial assistance.¹³² Similar initiatives in states like Kentucky and New York have shown that investing in digital infrastructure and provider training leads to increased telehealth usage, better patient engagement, and lower rates of medical debt in rural communities.¹³³ By amending Bill H.101 in this way, Massachusetts would set a higher standard for digital equity and healthcare access in rural areas.

Background

This bill is an effort to close the digital divide for some of Massachusetts's most vulnerable residents, who are those living in public housing. It directs the Department of Public Health to investigate how broadband internet access impacts public health goals, and it lays the groundwork for providing free, high-speed internet to all public housing developments in the state. The bill recognizes that reliable internet is now necessary for accessing telemedicine, educational resources, job applications, and finances. By focusing on public housing, the legislation aims to reach low-income families, seniors, and people with disabilities, who are left behind far too often in our current digital age. If enacted, the bill would require the state to fund

<https://www.countyhealthrankings.org/strategies-and-solutions/what-works-for-health/strategies/broadband-initiatives-for-unserved-and-underserved-areas>.

¹³² "Telehealth and Health Information Technology in Rural Healthcare Overview." 2025. Rural Health Information Hub. <https://www.ruralhealthinfo.org/topics/telehealth-health-it>.

¹³³ Michel, Jennifer. 2025. "Massachusetts Governor Announces \$31.5 Million for Public Housing Internet." Broadband Breakfast. <https://broadbandbreakfast.com/massachusetts-governors-announce-31-5-million-for-public-housing-internet/>.

and implement broadband infrastructure, ensuring that every resident in public housing can get online without cost barriers. This would be a concrete step toward digital equity, with the potential to improve health outcomes and economic opportunities for thousands of Massachusetts residents currently facing dangerous consequences. A case study demonstrating its efficacy can be seen in Kentucky.¹³⁴ In 2017, the Federal Communications Commission and the National Cancer Institute partnered with the University of Kentucky Markey Cancer Center to create the L.A.U.N.C.H. (Leading and Uniting National Communities in Health) Collaborative, a groundbreaking initiative designed to leverage broadband connectivity to create an "ecosystem of care" around rural patients. The project focused on improving cancer-related outcomes in underserved Appalachian areas by deploying cutting-edge technologies, including sensors, artificial intelligence, and mobile health technologies, to bridge gaps created by time and distance while promoting actionable intelligence for clinical care teams. This human-centered design approach involved community-driven protocols and trained healthcare professionals on how to integrate digital tools into their practice effectively. By 2019, 65% of rural hospitals in Kentucky had adopted telehealth services, and 61% had adopted Remote Patient Monitoring technologies, surpassing the national average for rural hospitals.

Amend and Enact Massachusetts Bill S.857, An Act to Ensure Uniform and Transparent Reporting of Medical Debt Data

Collecting geographical data on patients who apply for financial aid would allow the Massachusetts Center for Health Information and Analysis (CHIA) to recognize geographic regions with high demand for health-related financial aid. Though this data would not provide

¹³⁴ Hesse, Bradford W. 2020. "Barn-Raising on the Digital Frontier: The L.A.U.N.C.H. Collaborative." *Journal of Appalachian Health* 2, no. 1 (January). 10.13023/jah.0201.02.

CHIA with inferential ability as to why geographic disparities in financial applications exist, the data would serve as a meaningful starting point for addressing geographic disparities. By knowing from where financial aid applications are originating, CHIA could address population traits common to geographic areas and develop tailored strategies to lower the necessity of financial aid in said regions. In summary, geographic data itself would not explain the roots of geographic disparities in financial aid applications, but this data—coupled with the additional data mandated to be collected in Bill S.857¹³⁵—could serve as a starting point for future studies that aim to identify causes and propose solutions to areas with high concentrations of financial aid applications.

Background

Currently, no federal or state law exists that requires hospitals to collect geographic data about patients who apply for financial aid. Bill S.857 requires every hospital in Massachusetts to offer free or discounted healthcare to patients based on income or medical hardship.¹³⁶ Additionally, if enacted, it would prohibit funding discrimination on the basis of immigration status.¹³⁷ The bill also alters requirements for hospital reporting: it requires that hospitals report i) number of financial aid applications received, approved, denied, and incomplete, ii) total charity care and HSN reimbursements, iii) amount of medical debt sold to buyers, and iv) data broken down by age, gender, race, ethnicity, language, and insurance status to the Center for Health Information and Analysis.¹³⁸ This data would allow for much greater government oversight of charity care programs across Massachusetts.

Cost-Benefit Analysis

¹³⁵ An Act to Address Medical Debt through Hospital Financial Assistance Reform, S.842, Massachusetts Senate 194, Title XVII, Chapter 118E Massachusetts General Laws, <https://malegislature.gov/Bills/194/S842/BillHistory>.

¹³⁶ S.842, Massachusetts Senate 194.

¹³⁷ S.842, Massachusetts Senate 194.

¹³⁸ S.842, Massachusetts Senate 194.

1. **Administrative cost:** Between $\$80,186 * 5 = \$400,930$ and $\$80,186 * 8 = \$641,488$
2. **Software development cost:** Between $\$2,000,000$ and $\$8,000,000$, depending on customization and development firm
3. **Recommended appropriation for current fiscal year:** $\$8,641,488$
4. **Recommended appropriation for future fiscal years:** $\$641,488$

While the majority of this bill burdens hospitals with the cost of providing discounted healthcare, software development will be necessary for the state of Massachusetts to properly collect the data detailed in the current bill and the proposed amendment to the bill. The development of a large-scale Electronic Medical Record system that is HIPAA compliant and contains analytical components typically falls between $\$2$ and $\$8$ million.¹³⁹ In order to oversee the proper collection and usage of patient data, an additional 5-8 administrative managers must be hired. The typical salary of an administrative manager is $\$80,186$ ¹⁴⁰ at the Massachusetts Department of Public Health.

Limitations and Implementation

The most prominent limitation to this policy recommendation is designing a large, anonymous data collection software that can be standardized and used across each hospital in the state of Massachusetts. Software development has only become a more lucrative industry in recent years with the rise in AI, so securing a company to develop a massive software at a reasonable price might pose a significant challenge to this policy recommendation. Additionally, developing such an expansive software could take multiple years following the appropriation of funds. The development of a mass-scale, state-run software will doubtlessly be complex, so

¹³⁹ Martin Luetic, “How Much Does EHR Software Development Cost? 7 Key Cost Factors Explained,” Decode, October 20, 2025, <https://decode.agency/article/ehr-software-development-cost/>.

¹⁴⁰ “Statewide Payroll Open Payroll,” Massachusetts Department of Public Health, accessed November 18, 2025, https://cthrupayroll.mass.gov/#/year/2022/full_time_employees.others/pay1/explore/0-0-0/position_title/Administrator+IV/0/departement_division.

making software training accessible will be absolutely necessary, given that data collection is mandated as part of this policy proposal. Finally, ensuring applicant data remains confidential and protected from potential cyber attacks or leaks is of the utmost importance, for a breach in privacy could result in immense liability for the state of Massachusetts. Cybersecurity oversight and proper maintenance of data storage are crucial to the success of this policy recommendation, but implementation of these measures might be more costly than initially expected.

Organizations such as Undue Medical Debt and Healthcare for All MA could potentially be useful in gathering grassroots support for a policy initiative such as this one.

Amend and Enact Massachusetts Bill H.968, An Act Requiring Reimbursement for the Costs of Providing Competent Interpreter Services, and Amend Chapter 111, Section 25J of the Massachusetts General Law

Currently, under Chapter 111, Section 25J of Massachusetts law, every acute-care hospital must provide “competent interpreter services” to connect emergency department (ED) and acute-care services to every non-English speaker.¹⁴¹ However, in practice, certain gaps prevail—having the right to an interpreter doesn’t necessarily guarantee the assistance of one in a timely or effective manner; the current legislation protects EDs and acute-care services, but not outpatient, primary care, or community health—the common primary care that Americans are receiving on a day-to-day basis. Thus, Section 25J ought to expand its reach to include these “competent interpreter services” at outpatient, primary care, and community health centers in addition to ED and acute-care facilities.

¹⁴¹ “General Law - Part I, Title XVI, Chapter 111, Section 25J,” malegislature.gov, n.d., <https://malegislature.gov/Laws/GeneralLaws/PartI/TitleXVI/Chapter111/Section25J>.

Bill H.968 should be amended to require reimbursement to those additional, aforementioned settings, as well as requiring certain assessments to be completed with thresholds/metrics that set performance standards for interpreter services (may manifest in the likes of turnaround time, certification, data reporting, monitoring).

Cost-Benefit Analysis

Although all acute-care hospitals and emergency departments are required by-law to have interpreter services, access has remained inconsistent.¹⁴² In a 2021 survey conducted by *The American Journal of Managed Care* with health equity experts at Massachusetts General Hospital (MGH) on 224 recently discharged patients saw that a majority of patients with limited English proficiency (LEP) did not receive access to medical interpreters as promised. Relatedly, only 25% of Hispanic patients and 57% of Black patients had access to any laptops or tablets to aid them in the communication process.¹⁴³ The following cost-benefit analysis will address the ongoing problem of hospitals and healthcare settings lacking adequate interpreter services, but it should be acknowledged that having interpreter services does not equate to a just distribution of said services—something that ought to be remedied in the future.

1. Cost: Interpreter on Wheels (IOWs):

- a. The following numbers are based on numbers from Southcoast Health (a community health system in New Bedford, Massachusetts) in FY 2015.¹⁴⁴ Southcoast Health utilizes the Stratus Video software.

¹⁴² “Interpreter Services at Health Care Facilities,” Mass.gov, 2025, <https://www.mass.gov/interpreter-services-at-health-care-facilities>.

¹⁴³ Winifred Tobin, “Patients May Face Barriers due to Race, Ethnicity and Language at Hospital Discharge,” Massachusetts General Hospital, July 21, 2021, <https://www.massgeneral.org/news/press-release/patients-may-face-barriers-due-to-race-ethnicity-and-language-at-hospital-discharge>.

¹⁴⁴ Bill Siwicki, “Hospital Taps iPad-Based Language Interpretation That Takes Half the Time of Phone Calls,” Healthcare IT News, 2018, <https://www.healthcareitnews.com/news/hospital-taps-ipad-based-language-interpretation-takes-half-time-phone-calls?>

- i. 42,195 interpretation requests / ~45 iPads = 938 requests per iPad per year
- ii. **Average Length of Remote Interpretation Call:** 8.5 minutes
- iii. **Cost per Minute of Straus Video:** \$1.95 to \$4.95 per minute
- iv. Assuming the upper end, **Average Cost of One Remote Interpretation Call:** \$42.08 per call
 - 1. $42.08 * 938 = \$39,471.04$ per iPad per year
 - 2. $\$39,471 * 45 \text{ iPads} = \$1,776,195$ on interpretation services per hospital per year

2. Benefits: More accurate care provided; decreased returns to the hospital

- a. In the same report from New Bedford's Southcoast Health, they found that patients who actually understood treatment plans and prescriptions (heavily correlated with having proper communication) were far less likely to be readmitted to the hospital within 30 days of discharge. Naturally, this varies case to case in terms of disease severity, but fewer hospital visit effectively saves money.
 - i. On average, it costs a hospital \$20.49 for a primary care visit, \$61.54 for a discharged emergency department visit, and \$124.26 for a general inpatient stay.¹⁴⁵
 - ii. Patients with language barriers typically spend anywhere from 0.5 to 2 additional days in the hospital.¹⁴⁶

¹⁴⁵ Phillip Tseng et al., "Administrative Costs Associated with Physician Billing and Insurance-Related Activities at an Academic Health Care System," JAMA 319, no. 7 (February 20, 2018): 691, <https://doi.org/10.1001/jama.2017.19148>.

¹⁴⁶ "The Cost of Language Barriers in Health Care: A Case for Translation Support | Care to Translate," Caretotranslate.com, 2025, <https://www.caretotranslate.com/news/the-cost-of-language-barriers-in-health-care-and-a-case-for-translation-support>.

- b. In a report from Care to Translate, a digital medical translator service much like Southcoast Health’s Stratus Video,¹⁴⁷ they found the following benefits:
 - i. Providers would save time: on average, 10-15 minutes are usually spent beforehand to communicate with non-native speakers.

Limitations and Implementation

As mentioned previously, hospitals are legally required to have translation services, particularly in the ED. Furthermore, this will require a large allocation of funds; however, once again, the increased benefits for patients and hospitals would deem this a valuable return on investment in the long run. A legislator to reach out to early on would be Representative Kate Hogan, Massachusetts State Representative for the 3rd Middlesex district, who filed Bill H.968 originally. Since taking office in 2009, Representative Hogan has served as Chair of the Joint Committee on Public Health—thus having extensive experience overseeing public health policy in Massachusetts—and worked on legislation regarding healthcare insurance to increase equity for Massachusetts residents.¹⁴⁸ With this background—especially with the highly relevant Joint Committee on Public Health, seeing as these policy recommendations all fall under their jurisdiction—Representative Hogan and her colleagues would be a valuable connection to make.

Seeing as language and communication are so deeply interwoven with community—particularly immigrants—we recommend partnering with related organizations for lobbying. In particular, the Massachusetts Immigrant and Refugee Advocacy Coalition (MIRA) works in policy analysis and advocacy for the immigrant and refugee communities of Massachusetts, and have dedicated numerous efforts towards expanding language access.¹⁴⁹

¹⁴⁷ Care to Translate, “The Cost of Language Barriers in Health Care: A Case for Translation Support,” Care to Translate, January 14, 2025.

¹⁴⁸ “Public Health – Representative Kate Hogan,” Repkatehogan.comhas, 2015, <https://www.repkatehogan.com/priorities/public-health-ah/>.

¹⁴⁹ “About Us,” MIRA Coalition, n.d., <https://miracoalition.org/about-us/>.

Amend and Enact Massachusetts Bill S.2125/H.3384, An Act Relative to Language Access and Inclusion

Expanding Bill S.2125/H.3384 to mandate the translation of hospital financial materials into all languages required for other government operations in Massachusetts would represent a transformative step toward counteracting medical debt in the state. Currently, the bill requires state agencies to provide interpretation and translation for the most common non-English languages in the state, including Arabic, Cape Verdean Creole, Chinese (Simplified and Traditional), French, Haitian Creole, Khmer, Korean, Portuguese, Russian, Spanish, Vietnamese, and any other languages deemed necessary¹⁵⁰. By explicitly requiring hospitals to translate all vital financial documents, such as billing statements, financial assistance applications, and debt collection notices, into these mandated languages, the legislation would ensure that immigrants and residents with limited English proficiency have necessary access to information about their healthcare costs and rights. This would help prevent misunderstandings, missed deadlines, and unnecessary medical debt, which disproportionately affect non-English speaking communities¹⁵¹.

Such a mandate would also bring hospital practices in line with the broader language access standards already being established for public-facing state agencies in Massachusetts. The bill's approach is modeled after successful language access policies in other sectors, which have demonstrated that providing translated materials and interpretation services leads to higher rates of benefit utilization and greater trust in public institutions¹⁵². For example, during the

¹⁵⁰ DiDomenico, Sal. 2024. "An Act Relative to Language Access and Inclusion." Mass Speaks. <https://massappleseed.org/wp-content/uploads/2025/03/2025-26-Language-Access-and-Inclusion-Bill-Fact-Sheet-11.pdf>.

¹⁵¹ Loch, Brandon. 2025. "Massachusetts Bill Seeks to Counter Federal Push to Cut Language Access." Slator. <https://slator.com/massachusetts-bill-counters-federal-push-to-cut-language-access/>.

¹⁵² Halder, Sweta. 2023. "Overview of Health Coverage and Care for Individuals with Limited English Proficiency (LEP)." KFF.

COVID-19 pandemic, the lack of translated health and financial information left many immigrant families unable to access relief programs or understand their medical bills. This resulted in significantly increased financial hardship¹⁵³. By requiring hospitals to meet the same language access standards as other government agencies, Massachusetts would set a new benchmark for inclusion and accountability, ensuring that all residents can protect themselves from medical debt.

Background

This bill tackles one of the biggest obstacles faced by immigrants and non-English speakers in Massachusetts: the lack of accessible information and services in their language¹⁵⁴. S.2125/H.3384 requires state agencies, including MassHealth and other major health programs that benefit many immigrants and people of color in our state, to provide interpretation and translation for the most common non-English languages in the state. Agencies must develop language access plans, hire full-time coordinators to oversee these efforts, and set up advisory boards with community members who are directly affected by language barriers. The bill also mandates that vital documents, websites, and public notices be available in multiple languages and accessible formats, so everyone can understand and use state services. By making language access a legal requirement, this proposal aims to ensure that immigrants and residents with limited English proficiency can get the healthcare, benefits, and information they need without confusion or exclusion.

A case study showing the relevance of this can be found in a 2023 study by the Federal Reserve Bank of Philadelphia focused on examining the impact of California's Hospital Fair

<https://www.kff.org/racial-equity-and-health-policy/overview-of-health-coverage-and-care-for-individuals-with-limited-english-proficiency/>.

¹⁵³ Gomez, Juan C. 2021. "Immigrant Families during the Pandemic: On the Frontlines but Left Behind." The Center for Law and Social Policy. <https://www.clasp.org/publications/report/brief/immigrant-families-pandemic-frontlines/>.

¹⁵⁴ Massachusetts General Court. Bill S.2125/H.3384: An Act relative to language access and inclusion. 2025. <https://malegislature.gov/Bills/194/S2125>.

Pricing Act.¹⁵⁵ The law, among many considerations, mandated that hospitals must translate their charity care and discounted payment policies, notices, and applications into each language group spoken by 5% or 1,000 persons, whichever is less, of the hospital's service population. The study found that the law's requirements, including expanded income eligibility for financial assistance and mandatory translation of financial documents, led to a 16.5% reduction in the probability of patients having medical debt collections between \$1 and \$1,000, and a 40% reduction in non-medical debt in the same range.

Amend and Enact Massachusetts Bill S.902, An Act Lowering Health Care Prices for Patients

Legislators within the Massachusetts General Court should amend Bill S.902—which attempts to lower health care prices for patients—to require inclusion of means-based deductibles and/or pre-deductible coverage in most (or ideally all) public and private state health insurance programs. Means-based deductibles allow those who are lower-income but not fully eligible for Medicaid to pay less for their deductible rather than pay the full price. Pre-deductible coverage provides access to medical services even if an individual has not paid their deductible, granting greater care access (e.g., preventative care and chronic disease management) without having to pay a high out-of-pocket cost.¹⁵⁶

Cost-Benefit Analysis:

According to a study by the American Heart Association, the health care costs of cardiovascular risk factors are projected to triple from 2020 to 2050, with the economic toll

¹⁵⁵ Antwi, Yaa. 2023. “I’ve Got 99 Problems But a Bill Ain’t One: Hospital Billing Caps and Financial Distress in California.” Federal Reserve Bank of Philadelphia. <chrome-extension://efaidnbmnnnibpcajpglclefndmkaj/https://www.philadelphiafed.org/-/media/frbp/assets/workin-g-papers/2023/wp23-20.pdf>.

¹⁵⁶ Michelle H. Moniz et al., “Association of Childbirth with Medical Debt,” *Obstetrics and Gynecology* 143, no. 1 (2024): 11-13, <https://pmc.ncbi.nlm.nih.gov/articles/PMC11014426/>.

increasing from an estimated \$400 billion to a whopping \$1334 billion. In addition, annual health care costs related to diagnosed cardiovascular conditions are estimated to reach nearly \$1490 billion in 2050. Further, labor force productivity losses are anticipated to decrease by nearly 54%.¹⁵⁷

In Massachusetts, the median annual deductible for residents is \$2,900; however, deductibles range from \$930-7000 per year, with 10 percent of residents reporting deductibles of at least \$7000. For a 4-person family living 300% of the federal poverty line, total annual income comes to about \$96,000. These added expenses are compounded by other living costs including health insurance premiums, which total around \$24,000 for the average family. Reports from a 2023 national survey found that 29% of participants skipped care because of cost-related concerns. Of those 29%, 54% had their health problems worsen as a result.¹⁵⁸

In Massachusetts, approximately 44.3% of residents live within 400% of the federal poverty line, totaling around 3.1 million.¹⁵⁹ If the Massachusetts state legislature were to subsidize a minimum of \$1000 dollars for each of these individuals, the costs would be \$3.1 billion. However, considering that the economic productivity costs of cardiovascular disease alone contribute to losses of what would be, in Massachusetts alone, considering their respective proportion of the US population, \$31.3 billion by the year 2050. It is important to emphasize that this cost-saving is with respect to chronic heart disease alone, and not the hundreds of other conditions and preventative diseases that also contribute to financial losses to the U.S. market and healthcare system.

¹⁵⁷ D.S. Kazi et al., "Forecasting the Economic Burden of Cardiovascular Disease and Stroke in the United States Through 2050: A Presidential Advisory From the American Heart Association," *Circulation* 150, no. 4 (2024): 89-101, <https://www.ahajournals.org/doi/10.1161/CIR.0000000000001258>.

¹⁵⁸ "2023 Health Care Cost Trends Hearing," Massachusetts Health Policy Commission, 2023.

¹⁵⁹ "Distribution of the Total Population by Federal Poverty Level (above and below 400% FPL)," KFF, 2023, <https://www.kff.org/state-health-policy-data/state-indicator/population-up-to-400-fpl/?currentTimeframe=0&sortModel=%7B%22collId%22:%22Location%22,%22sort%22:%22asc%22%7D>.

Limitations and Implementation:

Considering that this recommendation is simply amending an existing bill in the Massachusetts state legislature, it should be allowed by the current statute. However, there are certain limitations to this intervention. Firstly, convincing representatives to apportion such a large sum of money without a promise of immediate reimbursement is a challenge. However, by emphasizing the importance of prioritizing community benefit over cost, we hope legislators will understand that this amendment is a long-term investment that is meant to restructure the current healthcare system, not provide an instantaneous and temporary fix.

Bill S.902 was originally presented by Senator Payano. To ensure that the aforementioned amendments are actually implemented into the final legislation, we recommend communicating and educating Senator Payano about how means-based deductibles and pre-deductible coverage can increase healthcare use and thus improve the health of Massachusetts residents. To aid in lobbying efforts, we recommend building an alliance with local healthcare advocacy-focused organizations. For instance, Health Care For All is a Massachusetts organization that prioritizes health justice and health equity across the state. By building these relationships with local groups, more pressure will be placed on the legislature to amend and pass S.902.

One potential actionable lobbying method is to direct employees at various advocacy organizations to schedule meetings with Senator Payano and other influential and key figures within the Massachusetts state legislature to discuss why these modifications are essential to reducing medical debt burden and consequently improving residents' health. Additionally, through engaging their volunteer and supporter bases, these advocacy organizations could set up community action events (e.g., phonebanking, letter writing, and signature gathering) to further pressure legislators to amend and pass S.902.

Amend and Enact Massachusetts Bill S.895, An Act to Promote Transparency and Prevent Price Gouging of Pharmaceutical Drug Prices

To address rising medical debt spurred by growing pharmaceutical costs, Massachusetts should implement S.895, which is a bill aimed at improving pharmaceutical pricing transparency legislation. The current legislation requires pharmaceutical companies to submit documentation explaining and supporting wholesale acquisition cost increases that surpass 15% in one year for brand-name drugs and price increases that surpass the evolving benchmark, as determined by a committee, for that year for generic and biosimilar drugs.¹⁶⁰ The bill also necessitates early notification for when new drugs hit the market and a 60-day notice of price increases. If a treatment's pricing is found to "substantially exceed" the drug cost, the bill calls for the company to enter into an access and affordability plan. There are fines for non-compliance; however, the bill emphasizes collaboration with pharmaceutical companies and increasing communication between the government and these companies over punitive measures.¹⁶¹ This bill should be amended to require price report submissions for all increases in wholesale acquisition costs that significantly outpace inflation, and not just those that lie above the benchmark. Additionally, and most importantly, these price reports should require pharmaceutical companies to disclose real transaction prices. With these amendments, passages of S.895 would help meaningfully address the rising pharmaceutical costs, thereby lowering costs for consumers and the accumulation of medical debt.

Background Research & Case Study:

The rise in the price of pharmaceuticals has been one of the largest contributors to the overall rise in total health care expenditures. Specifically, the pharmaceutical industry

¹⁶⁰ "Bill S.895," accessed October 20, 2025, <https://malegislature.gov/Bills/194/S895>.

¹⁶¹ "Bill S.895," accessed October 20, 2025, <https://malegislature.gov/Bills/194/S895>.

experienced a \$1 billion increase in pharmacy spending on rebates.¹⁶²¹⁶³ This rise in total health care expenditures contributes to the mounting medical debt issue by increasing prices that consumers and insurance companies fail to cover. Additionally, the increase in pharmaceutical costs significantly outpaces inflation, pointing to price-gouging as another explanation for the rise in costs, which price transparency legislation can help address.

This is evidenced first by a case study of California's pharmaceutical price transparency legislation, SB 17, which was seen to be somewhat effective. SB 17 was a bill passed in California that aimed to mitigate pharmaceutical costs by increasing pricing transparency with pharmaceutical companies.¹⁶⁴ Specifically, SB 17 aimed to do this by providing a 60-day notice for increases in drug prices. They set a 16% threshold for proposed and cumulative price increases in wholesale drug prices for reporting. They also had businesses include their reason for the price increase. As shown, the language of SB 17 is very similar to the proposed S.895 in Massachusetts. A case study of SB 17 highlights and pitfalls of this legislation while comparing it to pharmaceutical transparency legislation in other states. After analyzing 166 drug pricing laws with transparency components, this report found only six states (Vermont, Oregon, Maine, Louisiana, Connecticut, and Nevada) with effective legislation, so these states are also commonly referenced as comparison points.

Overall, the report found several pitfalls/oversights of SB 17 and even other states' legislation. However, there were some highlights. First, in the 3 years since the introduction of SB 17, there were decreases in the percent increase of wholesale acquisition costs for wholesale

¹⁶² "Health Care Spending Grew Faster Than Inflation, Labor Costs, and Income, Exacerbating Affordability Challenges for Massachusetts Residents | Massachusetts Health Policy Commission," accessed October 20, 2025, <https://masshpc.gov/news/press-release/health-care-spending-grew-faster-inflation-labor-costs-and-income-exacerbating> "Health Care Spending Grew Faster Than Inflation, Labor Costs, and Income, Exacerbating Affordability Challenges for Massachusetts Residents | Massachusetts Health Policy Commission." {Updating}

¹⁶³ "Community Hospitals at a Crossroads: Findings from an Examination of the Massachusetts Health Care System."

¹⁶⁴ Tran, Prescription Drug Pricing and Cost Transparency in California.

drugs.¹⁶⁵ Additionally, the median wholesale acquisition cost percent increases were a little lower for that same quarter the following year. This indicates some effectiveness in regard to transparency legislation. Additionally, the advance notices for price increases were somewhat helpful to health plans. They appreciated the advance notice, but also noted that the increase they were given was rarely ever truly reflected in the price they ended up receiving because of contractual language. Moreover, consequential drug prices tend to have consistent and predictable increases, which decreases the value of the early notice. Finally, some health insurers believe that this legislation, coupled with increased awareness and publicity, has helped moderate runaway prices in pharmaceutical costs. However, many drugs, especially gene therapies and specialty drugs, continue to be very expensive.

While the legislation had some benefits, the review pointed out many significant drawbacks that limit the effectiveness of this legislation. Firstly, this legislation, and all current drug price transparency legislation, fail to require the release of real transaction prices at each stage of the pharmaceutical distribution process. This information is vital for determining the actual value of medication and the price of production. Also, it is currently non-public information that would provide more insight into the pharmaceutical industry and pricing. One major challenge of SB 17 was acquiring actual new information. Since the bill was extraordinarily flexible, allowing companies to essentially give as much detail as they wanted to, leading to over 2/3rds of reports not containing any information that was not already available in the public domain.

Legislation like this can also have the inadvertent effect of bolstering the introduction of high-cost drugs. To retain profits and offset some battles with the threshold, companies may

¹⁶⁵ Tran, *Prescription Drug Pricing and Cost Transparency in California*.

simply introduce their drug at a higher cost, which defeats the purpose of price increase transparency legislation.

This legislation also misses a much-needed emphasis on the cost of specialty drugs. In 2020, specialty drugs accounted for only 1.6% of the drugs dispensed; however, they were 60.2% of the overall spending on prescription medication. This bill does not do much to help with that because of the vague language and lack of focus on specialty drug prices and price increases. Finally, the report emphasizes the need for more action coupled with transparency. Transparency is great in that it can help mitigate prices and direct legislators on where to focus their policy, but without specific action, the issue will persist.¹⁶⁶

Another report had similar findings. Ryan and Sood conducted an analysis of existing pharmaceutical pricing transparency legislation. The conclusions of this paper are similar to the last in that they both emphasize the need for pharmaceutical companies to disclose transaction prices as one of the most helpful health transparency efforts.¹⁶⁷ Both papers also emphasize the need for transparency laws to require the release of more relevant and useful information. They find that the list price usually does not reflect the price ultimately paid by consumers because of the confluence of other factors. They find that the drug manufacturer's net price is a more accurate representation of the price paid after rebates, etc. This paper goes more in-depth, calling most transparency laws "uninformative." They argue this because no current transparency laws require the release of real transaction prices/profits across all distribution system segments, as well as the failure to require transparency from more than 3 out of 5 segments of the pharmaceutical distribution system. (The 5 segments being wholesalers, manufacturers, insurers, PBMS, and pharmacies.) This paper emphasizes the lack of action taken on extraordinarily high

¹⁶⁶ Tran, *Prescription Drug Pricing and Cost Transparency in California*.

¹⁶⁷ Ryan and Sood, *State Drug Pricing Transparency Laws; Numerous Efforts, Most Fall Short*.

prices. Noting that in Vermont, thousands of drugs have been identified above the threshold, however, only 10 are being investigated.¹⁶⁸

Therefore, with the introduction of pharmaceutical price transparency legislation in Massachusetts, legislators need to ensure that these companies are disclosing useful information (through the disclosure of real transaction prices) and that the body responsible for investigating potential pharmaceutical price gouging has the capacity to pursue action in instances of price gouging.

Cost-Benefit Analysis:

1. Costs: Employee Wages

a. Administrative Employees

i. **Median Annual Salary:** \$89K¹⁶⁹

ii. Assuming 52 weeks and 40 hours per week, **Hourly Wage:** \$43

b. Legal Team

i. **Median Annual Salary:** ~\$110K¹⁷⁰

ii. Assuming 52 weeks and 40 hours per week, **Hourly Wage:** \$53

c. Note: The labor force will have to be sufficiently large to adequately address claims; therefore, estimates of labor costs should trend higher.

2. Benefit: Saving Constituents and Hospitals Money

a. Case studies have shown that introducing pharmaceutical pricing transparency legislation has reduced the number and magnitude of price increases. Ultimately, this will save patients and hospitals money as one of

¹⁶⁸ Ryan and Sood, State Drug Pricing Transparency Laws; Massachusetts Numerous Efforts, Most Fall Short.

¹⁶⁹ Glassdoor, “Commonwealth of Massachusetts Government Administration Salaries in United States Of America.”

¹⁷⁰ ZipRecruiter, “Salary.”

their most expensive margins – drug prices – should not be growing significantly year to year. This can especially help community health centers stay afloat independently, reducing the amount of money the state government has to invest to sustain these centers.

Limitations:

Implementing the most effective version of this bill would be highly unlikely, as there are major stakeholders who oppose the adoption of legislation that requires pharmaceutical companies to divulge real transaction prices. In the aforementioned report by Ryan and Sood, they note that pharmaceutical companies have continuously presented legal challenges to legislation like this in the past and would likely hinder the passage of legislation requiring the disclosure of real transaction prices.¹⁷¹ Therefore, realistically, the version of this bill that would be passed would not require that disclosure, even though it would be the most effective metric for ensuring real pharmaceutical drug pricing transparency. However, despite missing this effective metric, this legislation would still likely have an impact without that amendment, as seen with SB 17. Additionally, in the past, legislation similar to this has suffered or failed due to the lack of action following the identification of high-cost medication/treatment. Therefore, in implementing, the organization processing the information on high-cost drugs would have to be persistent, continually following through on conversations with pharmaceutical companies to bring costs down.

¹⁷¹ Ryan and Sood, *State Drug Pricing Transparency Laws; Massachusetts Numerous Efforts, Most Fall Short*.

Amend and Enact Massachusetts Bill H. 3264, An Act relative to payments in lieu of taxation by organizations exempt from the property tax.

House Bill 3264 proposes that municipalities can vote to mandate large tax-exempt organizations with a property value of greater than 15 million to make PILOT (Payment in Lieu of Taxes) payments instead of taxes, equal to 25% of what their property tax would have been if it were taxed.¹⁷² PILOT payments go directly to local governments. It also specifies that High Public Payment hospitals (hospitals where 63% of gross patient service revenue is from public payers such as Medicare and Medicaid) are exempt from this proposition.¹ Importantly, community benefits can be an alternative form of a PILOT payment.¹ Through this, the bill sets minimum requirements for hospitals to use tax-exempt dollars for equitable purposes, either by enabling local governments to utilize tax-exempt dollars for the public benefit or by pushing the hospitals to directly utilize the money towards community benefits such as free or discounted care. However, rather than simply requiring 25%, the minimum requirements for hospitals should equal or exceed the full amount of state property tax exemption that a state would have received to go towards payments to local governments or community benefits in order to make a greater impact towards equitable use of tax-exempt dollars and ultimately aid in reducing medical debt. This bill should additionally enforce minimum requirements by granting explicit authority to local governments for the collection of payments to be pursued through court collection efforts after a certain time frame, in which the hospital is required to pay all costs associated with these court collection efforts.

Background - Tax Exemption and Community Benefits

¹⁷² “Massachusetts H3264 | 2025-2026 | 194th General Court.” LegiScan, 2025, legiscan.com/MA/text/H3264/id/3168379. Accessed 21 Oct. 2025.

Though tax-exempt hospitals are meant to use their exemptions towards free and reduced care for low-income populations, the total estimated value of tax exemptions in 2020 in the US is around 28 billion dollars, greatly exceeding the total estimated charity care of only 16 billion dollars by nonprofit hospitals.¹⁷³ A study on 2018 Medicare cost reports also suggests that nonprofit hospitals devote a similar percentage of their total expenses towards charity care and unreimbursed Medicaid costs (which constituted most of the community benefit costs in 2017) compared to for-profit hospitals.¹⁷⁴ Furthermore, the government accountability office's 2020 analysis of IRS data found that there were 30 hospitals that didn't report any spending on community benefits in 2016.¹⁷⁵ In addition, despite mandatory reporting, in the last ten years, the federal IRS has not used its authority to enforce financial assistance standards in nonprofit hospitals by threatening to revoke their nonprofit status.⁴ Resultantly, there is a lack of federal regulation on the tax exemption status of nonprofit hospitals. While states have no jurisdiction over federal nonprofit status, states have clear constitutional authority over state property tax exemptions—an area in which policy can play a role in ensuring that the tax-exempt money goes towards alleviating medical burdens on the community, particularly low-income and underserved populations. It does not make sense for nonprofit hospitals to receive a tax exemption if their contributions to charity care and other community benefits are not increasing compared to those of for-profit hospitals.

Community benefits can be defined as six various possibilities, including: 1) operating an emergency room available to anyone regardless of if they can pay, 2) having a board of

¹⁷³ Hulver, Scott. 2023. "The Estimated Value of Tax Exemption for Nonprofit Hospitals Was about \$28 Billion in 2020 | KFF." KFF. March 14, 2023. <https://www.kff.org/health-costs/the-estimated-value-of-tax-exemption-for-nonprofit-hospitals-was-about-28-billion-in-2020/>.

¹⁷⁴ Bruch, Joseph D., and David Bellamy. 2020. "Charity Care: Do Nonprofit Hospitals Give More than For-Profit Hospitals?" *Journal of General Internal Medicine* 36 (10). <https://doi.org/10.1007/s11606-020-06147-9>.

¹⁷⁵ "United States Government Accountability Office." 2023. <https://www.gao.gov/assets/gao-23-106777.pdf#page=10>.

independent civic directors from the community, 3) having an open medical staff policy, 4) providing care for anyone able to pay without discrimination based on factors such as using Medicare or Medicaid, and 5) using funds to improve facilities, equipment, and patient care or 6) to improve training, education and research.¹⁷⁶ An issue is that there are no standardized requirements for what has to actually be met in the broad range of possible ways of compliance, nor is there any enforcement on compliance, as seen by how some hospitals reported no spending on community benefits.

Cost-Benefit Analysis

Costs: Implementation of Bill H. 3264 would mostly involve costs at the municipal level rather than the state level. Municipal governments would need to identify tax-exempt hospitals that have property values greater than \$15 million, calculate the property tax liability, determine whether hospitals meet the minimum requirements through PILOT payments or community benefit expenditures, and pursue court collection efforts if payments are not made within the required time frame. These responsibilities could be handled by existing municipal assessors' offices and legal departments, limiting the need for significant new staffing. For instance, the Municipal Assessor's office already works to determine property values and tax liabilities and can be responsible for identifying tax-exempt hospitals with property values greater than \$15 million and calculating the hospital's estimated property tax liability, while the municipal treasurer can receive and track PILOT payments made by hospitals since they already manage municipal fees and revenues.^{177 178}

¹⁷⁶ "United States Government Accountability Office." 2023.
<https://www.gao.gov/assets/gao-23-106777.pdf#page=10>.

¹⁷⁷ "The Role of the Assessor in Municipal Finance." 2022. Mass.gov. 2022.
<https://www.mass.gov/info-details/the-role-of-the-assessor-in-municipal-finance>.

¹⁷⁸ "The Role of the Treasurer in Municipal Finance." 2024. Mass.gov. 2024.
<https://www.mass.gov/info-details/the-role-of-the-treasurer-in-municipal-finance>.

Benefits: According to the Lown Institute, Massachusetts hospitals received around \$1.7 billion in tax benefits annually from 2020–2022, while spending only \$990 million on community benefits, resulting in a fair share deficit of approximately \$710 million per year.¹⁷⁹ Bill H. 3264 would allow municipalities to recover a part of this deficit either through PILOT payments that directly support local public services or through required community benefit spending such as free or discounted care. If the bill is amended so that minimum requirements equal or exceed the full amount of the state property tax exemption rather than only 25%, the potential recovered value would be substantially higher, but even a partial recovery of the annual \$710 million fair share deficit would generate revenue far more than any additional municipal administrative costs (if any) while also ensuring that tax-exempt dollars are either reinvested locally or directly reduce medical costs for patients.

Limitations and Implementations

This recommendation is permissible under current Massachusetts law since the property tax authority is explicitly attributed to the Commonwealth of Massachusetts as stated in Massachusetts General Laws Chapter 59, and this recommendation remains under State jurisdiction, not extending to federal nonprofit status under IRS §501(c)(3).¹⁸⁰ Legislative support would involve members of the Joint Committee on Revenue, which oversees property tax exemption and PILOT legislation, including House Bill 3264 (2025–26 session).¹⁸¹ Working with the bill sponsors and presenters, such as Erika Uyterhoeven, Christine P. Barber, and Natalie M. Higgins, is important to incorporate amendments into the existing bill.¹¹ County and municipal governments would also be likely allies since they are the ones that have to deal with

¹⁷⁹ “Making the Hospital Tax Exemption Work for Massachusetts.” n.d. Accessed October 13, 2025. https://lownhospitalsindex.org/wp-content/uploads/2025/04/Massachusetts_state-report.pdf.

¹⁸⁰ “General Law - Part I, Title IX, Chapter 59, Section 5.” 2025. Malegislature.gov. 2025. <https://malegislature.gov/Laws/GeneralLaws/PartI/TitleIX/Chapter59/Section5>.

¹⁸¹ “Bill H.3264.” 2025. Malegislature.gov. 2025. <https://malegislature.gov/Bills/194/H3264>.

the impact of tax exemption on large hospitals while continuing to provide essential services and infrastructure. The Massachusetts Municipal Association (MMA), which advocates for municipal affairs, is also a potential partner.¹⁸²

A potential limitation is that community benefits are a loosely defined term, which could enable hospitals to claim compliance through low-impact activities, making minimal impacts towards alleviating medical debt. To combat this, additional regulations need to clearly define qualifying community benefits, and standardized methods need to be created to calculate the value of community benefits.

Additionally, enforcement relies heavily on municipal action in pursuing court collection efforts. Variance in municipal action towards these efforts could impede the successful implementation of these policies. Therefore, clear timelines, explicit enforcement authority, and standardized procedures would be essential to ensure that hospitals cannot delay or avoid compliance. Exemptions for High Public Payment hospitals might also limit the impact of this policy in places where hospitals rely heavily on public payers, reducing the total amount of revenue or community benefit spending able to be recovered in those locations.

Enact Legislation Modifying the Massachusetts General Laws

Enact a novel regulation in Massachusetts that implements penalties or publicizes lists of hospitals noncompliant with price transparency regulations (HB 1285)

Massachusetts legislatures should enact a novel regulation modeled after Colorado's HB 1285, which prohibits certain medical debt collection activities when hospitals fail to comply

¹⁸² "History and Mission." n.d. Massachusetts Municipal Association (MMA). <https://www.mma.org/about-mma/mma-history-and-mission/>.

with price transparency requirements and allows patients to seek relief in court.¹⁸³ Taking such an approach builds off of existing mandates, like those in the federal No Surprises Act and Massachusetts' hospital price reporting rules¹⁸⁴. Specifically, it would entail constructing a tiered penalty structure, starting with warnings and fines for initial non-compliance and then further escalating for repeat offenses, with revenues directed to a patient relief fund or medical debt mitigation as a policy design choice rather than a currently enacted framework in either state. Noncompliant facilities would, in turn, be listed prominently on a state-run online dashboard, which would be made accessible via mass.gov, and their violations would be listed along with estimated patient impact, compliance timelines, and any other relevancies similar in function to Colorado's Hospital Price Transparency Compliance website.¹⁸⁵ These measures would be expected to incentivize hospitals to adhere to real-time pricing disclosures for shoppable services and would also empower consumers, namely low-income and chronically ill patients who are the most vulnerable to surprise billing, to compare costs and reduce their risk of medical debt, consistent with broader evidence that price transparency and surprise billing protections can lessen financial exposure. Colorado has shown compliance improvements after the enactment of HB 1285, including state evaluations reporting that a majority of hospitals now post required

¹⁸³Cooke, John, Dominick Moreno, Daneya Esgar, and Patrick Neville. n.d. "Prohibit Collection Hospital Not Disclosing Prices." <https://leg.colorado.gov/bills/hb22-1285>.

¹⁸⁴ "The No Surprises Act Has Reduced Patients' Out-of-Pocket Spending for Medical Care | Mass General Brigham." 2025. [Massgeneralbrigham.org](https://www.massgeneralbrigham.org/en/about/newsroom/press-releases/no-surprises-act-reduced-patients-out-of-pocket-spending). August 27, 2025.

¹⁸⁵ new, Colorado's. 2019. "PatientRightsAdvocate.org." [PatientRightsAdvocate.org](https://www.patientrightsadvocate.org/coloradotransparency). 2019.

machine-readable files.¹⁸⁶ This suggests that Massachusetts could achieve similar gains if there is an implementation of a bill very similar to HB 1285.^{187,188}

Case Study:

Colorado's HB 1285 prohibits debt collection during hospital non-compliance periods, allowing for patients' lawsuits and requiring state oversight with public reporting. In November 2022, early state evaluations showcased that only a minority of Colorado hospitals were fully compliant with machine-readable and shoppable service file requirements. This resulted in an increased rate of surprise billing risks for low-income patients under 200% FPL facing opaque chronic care costs like diabetes management. This has been consistent with national evidence that surprise bills and opaque pricing burden lower-income and medically vulnerable patients. After the bill went into place, the Colorado Department of Health Care Policy and Financing evaluations showed that in February 2025, 45.6% of hospitals reviewed had an overall quality rating of "Good," while 18.4% were rated "Poor," and there was 60% compliance in a 35-hospital sample compared to 21.1% national compliance. Put simply, following the bill, Colorado has been outperforming national benchmarks on hospital price transparency. Later in August of that year, there was a dip to 39.6% "Good" and 40.6% "Poor" because of CMS updates on negotiated rates and related federal file requirements, but Colorado still managed to rank 8th nationally with 65.4% of hospitals receiving the highest Turquoise Health, five stars¹⁸⁹. On another note, HB 1285 reduced the potential of financial harm by restricting debt collection

¹⁸⁶ "Hospital Price Transparency | Colorado Department of Health Care Policy & Financing." n.d.

Hcpf.colorado.gov. <https://hcpf.colorado.gov/hospital-price-transparency>.

¹⁸⁷ H, Rex. 2022. "HB22-1285: Colorado Hospital Price Transparency Law." Mdclarity.com. December 14, 2022. <https://www.mdclarity.com/blog/hb22-1285-colorado-hospital-price-transparency>.

¹⁸⁸ "Out-of-Network Rate Recommendations_090821.Pdf | Mass.gov." 2021. Mass.gov. 2021.

<https://www.mass.gov/doc/report-to-the-massachusetts-legislature-out-of-network-rate-recommendations>.

¹⁸⁹ Colorado Department of Health Care Policy & Financing. Hospital Price Transparency Posting Evaluation Report: Spring 2025. April 2025.

[https://hcpf.colorado.gov/sites/hcpf/files/Price%20Transparency%20Postings%20Evaluation%20Report%20\(April%202025\).pdf](https://hcpf.colorado.gov/sites/hcpf/files/Price%20Transparency%20Postings%20Evaluation%20Report%20(April%202025).pdf).

during non-compliance and supporting patient lawsuits with penalties up to the amount of the debt and attorney's fees. HCPF also noted that there was notable progress towards shoppable care access through improved federal price transparency postings, with Spring 2026 valuations expected to be even better as hospitals adapt even more to 2025 CMS guidance.

Cost-Benefit Analysis:

Hospital price opacity is associated with widespread surprise billing and medical debt; surprise billing has been estimated to reduce commercial insurance premiums by 1.6–5.1% and save roughly \$12–\$38 billion annually for the commercially insured population. In Massachusetts, out-of-network billing analysis shows that the average potential balance bill was about \$167 per claim, with amounts ranging from \$5 at the 5th percentile to \$749 at the 95th percentile, which clearly delineated how surprise charges can quickly accumulate for patients.¹⁹⁰ Based on the implementation of the federal No Surprises Act, robust surprise billing protections have been shown to reduce patients' out-of-pocket spending by nearly \$600 per privately insured adult per year. In other words, better price transparency means savings of thousands for families. Using the fundamentals of Colorado's HB 1285, which act to prohibit pursuit of patient debt for hospitals that fail to meet price transparency requirement (and also enables patients to recollect any paid money back from a noncompliant hospital) if in Massachusetts there were efforts of combining tiered penalties, generating public non-compliance dashboards, and installing debt-collection bans, this would on the order of a few million dollars per year in administrative and enforcement resources, but at the same time, saving on the order of hundreds of millions of dollars annually through reductions in surprise bills and other related cost shifting. These modeled savings give assurance that stronger transparency and enforcement in Massachusetts

¹⁹⁰ Huang, Yue, Laura Nasuti, and David Auerbach. 2021. "Out-Of-Network 'Surprise Billing' in Massachusetts: Estimating the Prevalence and Financial Impact." *Health Services Research* 56 (S2): 12–13. <https://doi.org/10.1111/1475-6773.13729>.

should lead to proper price shopping and contract discipline, like what has been observed when surprise billing protections and transparency rules reduce out-of-network exposure and out-of-pocket spending elsewhere. In this case, low- and moderate-income residents who are often faced with high deductibles and have garnered a lot of medical debt would be expected to be most benefited from improved price comparability and reduced risk of collections on noncompliant hospital bills.¹⁹¹

Limitations and Implementation

Enacting penalties and public lists for hospital price-transparency non-compliance in Massachusetts, mirroring Colorado’s HB 1285, despite being modeled off existing mandates like Massachusetts’ Hospital Charge Book, faces a limitation related to enforcement capacity. Colorado’s HCPF required dedicated staffing to conduct compliance evaluations, and its 2023 statewide review found that only 26.5% of hospitals and 31.1% of hospital systems met “Good” compliance standards, while critical access hospitals showed just 15.6% compliance.¹⁹² However, Massachusetts’ Department of Public Health could lack this capacity if there aren’t any new appropriations. Furthermore, hospitals could challenge back using fines via lawsuits, which would end up delaying any positive impact of alleviating medical debt. To enact this bill, the first steps of implementation would entail targeting the DPH authority for tiered penalties and debt-collection bans during violations, and to update theMass.gov dashboard in a similar fashion to Colorado’s. Further, partnership with sponsors like Senator Julian Cyr, who advocated for transparency in the 2022 Resilient Health Care Act, while submitting a testimony with HCPF

¹⁹¹ Duffy, Erin, Erin Trish, and Loren Adler. 2020. “Surprise Medical Bills Increase Costs for Everyone, Not Just for the People Who Get Them.” Brookings. October 2, 2020. <https://www.brookings.edu/articles/surprise-medical-bills-increase-costs-for-everyone-not-just-for-the-people-who-get-them/>.

¹⁹² Colorado Department of Health Care Policy & Financing. Hospital Price Transparency Posting Evaluation Report: Spring 2025. April 2025. [https://hcpf.colorado.gov/sites/hcpf/files/Price%20Transparency%20Postings%20Evaluation%20Report%20\(April%202025\).pdf](https://hcpf.colorado.gov/sites/hcpf/files/Price%20Transparency%20Postings%20Evaluation%20Report%20(April%202025).pdf).

data showing Colorado’s compliance rise among hospitals subject to HB 1285 from 18–30% pre-2023 to 33.3% in the 2023 evaluation, is also very important. Another effort is coalitions, which can be built with Massachusetts advocacy groups like Health Care For All, which lobbied S.902 for equity, and with the Massachusetts Health & Hospital Association. Constituent meetings could be scheduled with Representative Kate Hogan via district offices, and phone banks can be organized through the thousands of supporters of the HCFA. As with many other movements for change to be brought, a petition can also be launched on Change.org to highlight the medical-debt risks facing vulnerable Massachusetts residents.

Appropriate Discretionary Funding for Marketing of Community Health Centers

Currently, community health centers in Massachusetts are struggling to continue operations, with many operating at a loss each year.¹⁹³ Community health centers are vital, especially considering the issue of rising medical debt, as they present a low-cost health option for constituents.¹⁹⁴ Funds are continually appropriated for the maintenance of community health centers. Most recently, as of September 2025, an additional \$234 million was appropriated in emergency funding for community health centers.¹⁹⁵ This comes in the wake of changes under the Trump administration due to the passage of the Big Beautiful Bill. While increasing funding for operations is a viable short-term solution, it does not address the issue of the consistent deficit that community health centers operate under. Therefore, Massachusetts should appropriate discretionary funding specifically for the marketing of community health centers in an effort to drive utilization by commercial payors. Approved marketing ventures would include the

¹⁹³ Santana, *Financial Margins At Mass. Hospitals Tightening, Or In The End* | SHNS | September 25, 2025 – *Health Care For All*, n.d.

¹⁹⁴ “Community Hospitals at a Crossroads: Findings from an Examination of the Massachusetts Health Care System.”

¹⁹⁵ Santana, *Financial Margins At Mass. Hospitals Tightening, Or In The End* | SHNS | September 25, 2025 – *Health Care For All*, n.d.

expansion of digital marketing, programs to improve patient retention, physician outreach, and more. With an increase in commercial payors, instead of solely those on Medicare/Medicaid, community health centers will bring in the profits necessary to continue operations and then go on to serve a wider array of people. Expanding this low-health-care-cost option for consumers will lead to a decrease in the amount of accumulated medical debt.

Background

A cycle starting with stigma continually pushes individuals away from utilizing community health centers.¹⁹⁶ The belief that community health centers provide worse care directs individuals toward more expensive private and specialty options. Doctors further reinforce the non-community health center bubble by only recommending patients to options outside of community health. As a result, the community health centers end up underfunded, as the main source of funding – commercial payers – continue to choose other options. Community health centers also continue to lose money on unpaid bills. These factors cause community health centers to fare poorly financially with untenable margins, and in some cases, these centers are forced to close. Additionally, this lack of funds causes problems in retaining providers, especially because they do not have the bandwidth to pay them as much.¹⁹⁷ While these centers struggle to effectively function, they can not compete for patients or publicly increase their value to their community, thereby reinforcing the stereotype that they provide subpar care.

This intervention targets the final step of the cycle, stopping it from repeating by increasing the competitive value of community health centers in the public eye through marketing ventures. Bettering the public perception of community health centers brings in more

¹⁹⁶ “Community Hospitals at a Crossroads: Findings from an Examination of the Massachusetts Health Care System.”

¹⁹⁷ “Community Hospitals at a Crossroads: Findings from an Examination of the Massachusetts Health Care System.”

commercial payers, thereby allowing them to continue operations and pay physicians more competitive wages. Another aspect of marketing can also include ventures aimed at bridging provider connections and looping in community health workers into the field.

This short case study of an academic health system found that investing more into the marketing department – allowing for a more targeted and effective marketing strategy – generated a \$3 million increase in joint replacement capture volume in just 9 months, as one example.¹⁹⁸ Overall, the health system experienced a cumulative \$25 million increase in revenue in 9 months due to marketing efforts.¹⁹⁹ This case study demonstrates that paying for effective marketing will eventually pay for itself. This study supports the claims of other articles which emphasize the benefits of investing in marketing for increasing visibility, bolstering hospital reputation, improving competitive advantage, and more.²⁰⁰

Cost-Benefit Analysis:

a. Cost per center:

- i. Typically, hospitals seeking growth appropriate around 8-10% of their revenue to marketing.^{201,202}

1. Total Revenue for Community Health Centers in Mass: \$67,759,441.²⁰³
2. Therefore, the Massachusetts legislature should invest at least \$5.4 million into marketing for community health centers.

b. Benefit: Increases the Number of Commercial Payors

- i. Even a conservative 2% increase in commercial payers could help community health centers bring their margins out of the negatives or back to earlier levels. In

¹⁹⁸ Sullivan, “Academic Health System Increases Patient Retention By 2.4% (~\$25M In Revenue) In 9 Months.”

¹⁹⁹ Sullivan, “Academic Health System Increases Patient Retention By 2.4% (~\$25M In Revenue) In 9 Months.”

²⁰⁰ Purcarea, “The Impact of Marketing Strategies in Healthcare Systems.”

²⁰¹ Marketing, “How Much Should Healthcare Leaders Spend on Marketing?”

²⁰² Barasa, “Strategic Healthcare Marketing Budgeting.”

²⁰³ “Massachusetts League of Community Health Centers | Boston, MA | Cause IQ.”

2025, community health center margins dropped to 0.6% as opposed to 2.2% in 2023.²⁰⁴ Effective marketing could help close this gap, allowing for a sustainable, low-cost health option. This would help prevent centers from closing and allow the Massachusetts government to save by not continually appropriating a massive amount of additional funds for community health centers. Most recently, an additional \$37.5 million was appropriated for community health centers by the Massachusetts legislature.²⁰⁵

Limitations:

Investing in marketing initiatives would require building up currently sparse or potentially non-existent marketing infrastructure (especially in the digital marketing space) and adopting a marketing scheme that is effective, which can be a delicate challenge when focusing on healthcare. This process will take time and may take a while to yield tangible results. Therefore, for a period of time, marketing appears wasteful in light of all the other areas in dire need of funding. However, marketing will be extraordinarily useful in building up the long-term sustainability of community health centers.

Amend the Massachusetts General Laws c.59, §5 to Mandate Annual Community Benefit Reporting

Setting minimum requirements is extremely important; however, to further enforce these requirements, mandating community benefit reporting, rather than making it a voluntary guideline, is essential. Doing so would improve Massachusetts' understanding of where

²⁰⁴ Santana, *Financial Margins At Mass. Hospitals Tightening, Or In The End* | SHNS | September 25, 2025 – *Health Care For All*, n.d.

²⁰⁵ “Governor Healey Signs Bill Providing Critical Funding for Hospitals and Community Health Centers in Massachusetts | Mass.Gov.”

tax-exempt hospitals' money is distributed, and publicizing such information can act to provide greater enforcement of tax exemption. Specifically, the requirement should be added to Massachusetts General Laws Chapter 59 Section 5—the property-tax exemption statute—either by amending Clause Three or by creating a hospital-specific exemption subsection for Clause Three that makes annual community benefit reporting a mandatory condition for tax exemption status, with proof of community benefit spending/alternative payments as a condition for retaining state property tax exempt status.²⁰⁶

Background - Reporting and Enforcement

Currently, hospitals across the US are federally required to “conduct a community needs assessment (CHNA) every three years and adopt an implementation strategy to meet the community health needs identified through the CHNA to the federal government’s IRS.”²⁰⁷ They also have to annually file Form 990 and attach Schedule H, which asks for total community benefit expenditures and total hospital expenditures and revenues to the IRS.²⁰⁸ Massachusetts currently does not require *state* community benefit reporting and only has voluntary guidelines for annual reporting.²⁰⁹ Implementing annual reporting to a state institution would not be difficult, considering that hospitals already do so on a federal level.

Additionally, Massachusetts has a state-managed program called the Health Safety Net

²⁰⁶ “General Law - Part I, Title IX, Chapter 59, Section 5.” 2025. Malegislature.gov. 2025. <https://malegislature.gov/Laws/GeneralLaws/PartI/TitleIX/Chapter59/Section5>.

²⁰⁷ “Community Health Needs Assessment for Charitable Hospital Organizations - Section 501(R)(3) | Internal Revenue Service.” Irs.gov, 2018, www.irs.gov/charities-non-profits/community-health-needs-assessment-for-charitable-hospital-organizations-section-501r3? Accessed 21 Oct. 2025.

²⁰⁸ “Instructions for Schedule H (Form 990) (2024) | Internal Revenue Service.” Irs.gov, 2024, www.irs.gov/instructions/i990sh.

²⁰⁹ “Community Benefit Requirement.” n.d. Accessed October 13, 2025. https://www.hilltopinstitute.org/wp-content/uploads/hcbp/hcbp_docs/HCBP_CBL_MA.pdf.

that provides financial assistance and also recommends, through voluntary guidelines, that nonprofit hospitals should have community benefits to receive state tax exemptions.²¹⁰ However, there are no mandatory minimum requirements for use of funds towards community benefit or charity care beyond the initial licensure requirements.¹⁷ The initial license (Mass. Gen. Laws ch. 111, § 51G(3)) requires nonprofit and for-profit hospitals to provide charity care.¹⁷ After this initial licensure, hospitals can keep their tax-exempt status without any enforcement mechanisms for actually using tax-exempt dollars towards community benefits. As a result, hospitals can easily retain exemption, regardless of whether their community benefit spending offsets the public costs of tax exemption status, with the state having no record or regulation of these hospitals' use of exemption status.

A study by the Lown Institute found that from 2020 to 2022, 58% of nonprofit hospitals in Massachusetts had a “fair share deficit”, in which they received more tax benefits than they actually spent towards community benefits and health.²¹¹ There was also a lot of inconsistency in reported spending by hospitals to the IRS versus to the state in 11 hospitals.

Therefore, it seems that greater enforcement of requirements for community benefit spending is essential through standardized reporting systems in which annual reporting is a requirement for state property tax exemption status. This can be very impactful in helping the issue of medical debt in Massachusetts by improving access to health services for low-income and underserved populations.

Cost-Benefit Analysis:

²¹⁰ “Community Benefit Requirement.” n.d. Accessed October 13, 2025.

https://www.hilltopinstitute.org/wp-content/uploads/hcbp/hcbp_docs/HCBP_CBL_MA.pdf.

²¹¹ “Making the Hospital Tax Exemption Work for Massachusetts.” n.d. Accessed October 13, 2025.

https://lownhospitalsindex.org/wp-content/uploads/2025/04/Massachusetts_state-report.pdf.

In Illinois, the Attorney General is responsible for monitoring the annual state reports by nonprofit hospitals and ensuring that they meet quotas. Massachusetts' Attorney General is currently Andrea Campbell.

Cost: At least 3 employees to aid in monitoring compliance by examining and organizing reports.

- 1 Senior Auditor — salary around \$70,654.35 - \$105,981.5/yr ²¹²
- 2 Staff Auditors — salaries around \$55,000.00 - \$65,000.00/yr ²¹³
- Thus, total costs would be \$200,000 per year.

Benefits: According to the Lown Institute, in 2020-2022, Massachusetts hospitals received \$1.7 billion in tax benefits every year. The money actually spent towards community benefits was around \$990 million. Thus, the fair share deficit was \$710 million. Overall, profits would then be around \$709,800,000, calculated by subtracting the fair share deficit from the potential costs of employees. The total amount of medical debt in Massachusetts in 2019-2021 was around \$768 million. The amount recovered from the fair share spending towards community benefits and improving healthcare for low-income and underserved communities is thus enough to eliminate around 92.42% of medical debt in Massachusetts. Though the money would not go directly towards medical debt, the indirect benefits through discounted services, preventive measures, education, etc. would certainly make impacts in the root of the system that causes continued medical debt accumulation in Massachusetts residents.

Limitations and Implementations

²¹² “Job Opening: SENIOR AUDITOR II.” 2024. Mass.gov. 2024.
<https://www.mass.gov/info-details/job-opening-senior-auditor-ii>.

²¹³ “Job Opening: FIELD AUDITOR I.” 2024. Mass.gov. 2024.
<https://www.mass.gov/info-details/job-opening-field-auditor-i#salary-range>.

This recommendation is permissible under current Massachusetts law with the amendment of the proposed statute. Property tax authority is explicitly attributed to the Commonwealth of Massachusetts as stated in Massachusetts General Laws Chapter 59, and this recommendation remains under State jurisdiction, not extending to federal nonprofit status under IRS §501(c)(3).²¹⁴ Using precedents in Illinois and Nevada, as well as Massachusetts' own precedent of tying hospital licensure to charity care obligations, there is clear authority for the legislature to similarly tie state property tax exemption status with compliance with minimum community benefit requirements through annual reporting. For successful implementation, however, it would require both statutory clarity as well as administrative coordination. As mentioned previously, enforcement could occur through the Attorney General's Office, and since hospitals already submit community benefit data to the IRS through Form 990 Schedule H, the added state-level reporting should not be too heavy a burden on hospitals, while working to improve accountability and transparency of nonprofit hospitals.

A major limitation of reporting-based approaches, as demonstrated by the Illinois case study, is that reporting alone does not guarantee increased community benefit spending or reduction of fair share deficits. Even with detailed reporting requirements, hospitals may continue to meet formal obligations without actually increasing charity care or reducing medical debt. This limitation highlights the importance of pairing reporting with minimum requirements for spending and enforcement mechanisms.

²¹⁴ "General Law - Part I, Title IX, Chapter 59, Section 5." 2025. Malegislature.gov. 2025. <https://malegislature.gov/Laws/GeneralLaws/PartI/TitleIX/Chapter59/Section5>.

Add a section to Massachusetts General Law, Chapter 1760, requiring health plans issued or renewed in Massachusetts to include a monthly income-adjusted limit on cost-sharing.

The MA Health Law Advocates—as an organization focused on protecting consumers from unaffordable medical debt—should lobby the adoption of a section requiring each and every health plan issued or renewed in Massachusetts to include a monthly income-adjusted limit on cost-sharing.²¹⁵ The section should be added onto M.G.L. c. 1760, a Massachusetts law enacted to impose consumer protections in health insurance with the aim to ensure fairness, transparency, and access to healthcare for all MA residents.²¹⁶ Adding a cap on monthly cost-sharing payments would align with the law’s intent by ensuring that Massachusetts residents can seek necessary healthcare without the risk of accumulating medical debt from high out-of-pocket costs.

Background Research and Case Studies

Patient cost sharing refers to the portion of healthcare costs that a patient must pay out of pocket.²¹⁷ These payments usually include things such as copayments, coinsurance, and deductibles, and were initially meant to exist so that patients can manage medical costs efficiently and use their healthcare resources in a more effective manner. However, in the state of Massachusetts, high patient cost sharing has become a prominent issue that is directly correlated with the accumulation of medical debt. The Massachusetts Health Policy Commission has found that the average annual cost sharing per person is on an upward trend in Massachusetts, with the cost growing from \$849 in 2019 to \$1,049 in 2023 (a 29% increase).²¹⁸ This rate of increase is

²¹⁵ “Health Law Advocates (HLA).” *Massachusetts Legal Aid*, 2026, masslegalaids.org/health-law-advocates-hla/. Accessed 1 Jan. 2026.

²¹⁶ “Mass. General Laws C.1760.” *Mass.gov*, 2020, www.mass.gov/lists/mass-general-laws-c1760. Accessed 3 Nov. 2025.a

²¹⁷ “Patient Cost Sharing | National Pharmaceutical Council.” *Npcnow.org*, NPC, 2024, www.npcnow.org/topics/patient-centered-formulary-benefit-design/patient-cost-sharing.

²¹⁸ “HPC Shares New Research on Improving Affordability and Predictability in Cost Sharing, Announces the Launch of New Pharmaceutical and Health Planning Offices | Massachusetts Health Policy Commission.”

Masshpc.gov, 5 June 2025, masshpc.gov/news/press-release/hpc-shares-new-research-improving-affordability-and-predictability-cost-sharing.

5% faster than the insurer-paid spending, which is at only 24%. This is problematic because low-income individuals often lack the flexibility to reduce their healthcare use appropriately, so higher cost-sharing can directly lead to worsened health and can ultimately drive up overall costs through greater reliance on inpatient and outpatient services.²¹⁹ Essentially, those with lower income will be unequally affected by these high patient cost-sharing prices, leading to exacerbated societal inequities and also an overall higher chance of medical debt.

Recent research supports the implementation of a monthly cost-sharing cap as an effective tool to reduce medical debt among insured patients in Massachusetts. A 2022 Boston University School of Public Health analysis found that setting a \$500 monthly limit on out-of-pocket spending could cut patients' highest monthly costs by more than half, providing substantial financial relief to those with chronic medical conditions or who encounter sudden, unexpected medical events.²²⁰ A stricter \$250 cap produced even greater reductions, particularly benefiting individuals with high-deductible health plans and lower incomes. By reducing the effects of large, unpredictable spikes in medical spending throughout the year, a monthly cap ensures that residents can access necessary care without delaying or forgoing treatment due to immediate financial barriers. A national study further illustrates how out-of-pocket caps influence financial burden; however, their findings show that annual caps alone offer limited relief, as most insured individuals never reach those thresholds.²²¹ Instead, lower or income-adjusted caps yield much more meaningful protection, especially for those with low

²¹⁹ Chandra, Amitabh, et al. "The Impact of Patient Cost-Sharing on Low-Income Populations: Evidence from Massachusetts." *Journal of Health Economics*, vol. 33, Jan. 2014, pp. 57–66, <https://doi.org/10.1016/j.jhealeco.2013.10.008>. Accessed 8 Apr. 2020.

²²⁰ Shafer, Paul R., et al. "Monthly Cost-Sharing Limits and Out-of-Pocket Costs for Commercially Insured Patients in the US." *JAMA Network Open*, vol. 5, no. 9, 15 Sept. 2022, p. e2233006, www.ncbi.nlm.nih.gov/pmc/articles/PMC9478767/, <https://doi.org/10.1001/jamanetworkopen.2022.33006>.

²²¹ Riggs, Kevin R, et al. "Impact of Out-of-Pocket Spending Caps on Financial Burden of Those with Group Health Insurance." *Journal of General Internal Medicine*, vol. 30, no. 5, 4 Dec. 2014, pp. 683–688, <https://doi.org/10.1007/s11606-014-3127-z>. Accessed 3 Sept. 2023.

incomes. Together, these studies demonstrate that a Massachusetts-specific monthly cost-sharing cap, aligned with the consumer protection goals of M.G.L. c. 176O, could meaningfully reduce medical debt risk while improving affordability and access to care.

Estimated Cost of the Above Implementation

The simplest way that this policy could be funded is directly from the Massachusetts General Fund. This could be achieved by reallocating a portion of funds within MassHealth, particularly given that the Massachusetts Senate recently approved a \$3.7 billion budget increase, with the majority of that increase directed toward MassHealth.²²² Another potential way to secure funds for this policy is simply through reallocating existing health care budget priorities, such as redirecting rebates and savings from insurer rebates.

Monthly Cost-Sharing Cap	Annual Cost
State administrative costs, including factors such as additional staff and new IT resources to draft and properly enforce this legislation (based on a similar insurance program run in Massachusetts, the Health Plan Rate Review) ²²³	≈ \$12.8 million
New data systems/monitoring infrastructure for factors such as income verification and to have an up-to-date monthly reporting system (also based on the Health Plan Rate Review Modernization, where several new systems were created to expand the health plan rate review program) ²²⁴	≈ \$5 million

²²² Press Room. (2025). Retrieved August 11, 2025, from Malegislature.gov website: <https://malegislature.gov/PressRoom/Detail?pressReleaseId=201>

²²³ Massachusetts Division of Insurance. *Enhancements to the Massachusetts Health Insurance Rate Review Practice: Promoting Transparency and Protecting Massachusetts Consumers*. www.mass.gov/doc/sec-1003-premium-rate-review-narrativepdf/download.

²²⁴ “Massachusetts Rate Review Grants Award List | CMS.” *Cms.gov*, 2024, www.cms.gov/ccio/Resources/Rate-Review-Grants/ma. Accessed 2 Jan. 2026.

Amend Massachusetts General Law, Chapter 6D (Health Policy Commission) to implement a section on the necessity of stronger pre-transaction reviews and conditional approvals of all major healthcare mergers or acquisitions.

Massachusetts General Law, Chapter 6D, related to the Health Policy Commission, an independent state agency in Massachusetts that is responsible for promoting system transparency and advancing policies to make healthcare more equitable to residents.²²⁵ By empowering the Health Policy Commission’s authority to conduct more rigorous pre-transaction reviews of major healthcare mergers and acquisitions—and granting the commission the power to issue conditional approvals, such as imposing price caps or transparency requirements—Massachusetts can more effectively regulate and contain rising healthcare costs.

Background Research and Case Studies

High medical prices set by the health system are a major issue in the state of Massachusetts, with Mass General Brigham (MGB), the state's dominant health system, setting prices at an average of 280% times the cost of what Medicare would require one to pay for the same service.²²⁶ This pricing is absurdly high even in the domestic context, with the national average of medical prices being 247% of Medicare, a number that MGB clearly exceeds. The problem of rising medical costs in Massachusetts continues to escalate. According to the 2025 Annual Report on the Performance of the Massachusetts Health Care System, total health care expenditures (also known as THCE, a measure that calculates resident health care costs by

²²⁵ “Chapter 6D.” *Malegislature.gov*, 2025, malegislature.gov/Laws/GeneralLaws/PartI/TitleII/Chapter6D. Accessed 3 Nov. 2025.

²²⁶ “Arnold Ventures | Massachusetts Confronts High-Priced Health System.” *Arnold Ventures*, 2022, www.arnoldventures.org/stories/massachusetts-confronts-high-priced-health-system.

adding up all of the premiums, copays/coinsurance, and deductibles that a consumer must pay) per resident reached approximately \$11,153 in 2025—an 8.6% increase from levels reported in 2022 and 2023.²²⁷ This surge far exceeds the 2025 cost growth benchmark of 3.6%, a standard established in 2012 by state leaders to address the growing issue of rising healthcare costs and ensure that spending growth remains in line with the overall economy.⁹⁻²²⁸

One of the main reasons for these high medical costs is believed to be provider consolidation. A study of Massachusetts from 2013 to 2017 revealed a clear relationship between provider consolidation and rising healthcare prices, finding that as physicians became vertically integrated with hospitals and large systems, prices increased by 2.1–12% for primary care physicians and 0.7–6% for specialists, with the largest price hikes occurring in the state’s most dominant systems.²²⁹ This pattern shows that when hospitals vertically integrate by buying or partnering with smaller physician practices, it gives large health systems more bargaining power, letting them charge higher prices without necessarily providing better quality care or greater access for patients. This finding mirrors national trends that show that consolidation diminishes market competition while raising costs for insurers and patients, contributing to the widening gap between high- and low-priced providers. Consequently, empowering the Massachusetts Health Policy Commission to regulate such healthcare consolidations would help control medical costs for consumers and reduce the burden of medical debt.

Past regulations on healthcare mergers have demonstrated success in containing costs and reducing financial burdens on consumers. In 2019, the Beth Israel Deaconess Medical Center

²²⁷ “Health Care Spending Grew Faster than Inflation, Labor Costs, and Income, Exacerbating Affordability Challenges for Massachusetts Residents | Massachusetts Health Policy Commission.” *Masshpc.gov*, 13 Mar. 2025, masshpc.gov/news/press-release/health-care-spending-grew-faster-inflation-labor-costs-and-income-exacerbating.

²²⁸ “Special Report.” *Chiamass.gov*, 2025, www.chiamass.gov/rising-health-care-costs-impact-residents.

²²⁹ Curto, Vilsa, et al. “Price Effects of Vertical Integration and Joint Contracting between Physicians and Hospitals in Massachusetts.” *Health Affairs*, vol. 41, no. 5, 1 May 2022, pp. 741–750, <https://doi.org/10.1377/hlthaff.2021.00727>. Accessed 5 May 2022.

and Lahey Health were permitted to merge into the Beth Israel Lahey Health System under the condition that the new entity could not increase prices by more than 3% annually for the next seven years.²³⁰ Without this price cap, total healthcare spending was projected to rise from approximately \$128.4 million to \$170.8 million—an increase that would have placed a greater burden of medical debt on Massachusetts residents.²³¹ As of June 5, 2025, the HPC is conducting a comprehensive review of the impacts of this merger.²³² Nonetheless, this case serves as an important precedent for how regulated healthcare mergers can help control medical costs and mitigate medical debt.

Similarly, in 2024, the Providence Healthcare System was required to refund nearly \$21 million to low-income residents of Washington State following an investigation by the state government that showed that Providence had charged unreasonable medical prices, often resulting in significant debt for vulnerable patients.²³³ Although this case did not involve a merger, it underscores the critical role of strong state oversight in holding large healthcare systems accountable for fair pricing. Massachusetts could draw from this example to strengthen its own regulatory mechanisms and further reduce the burden of medical debt on its residents.

Estimated Cost of the Above Implementation

Since this policy falls within the Health Policy Commission (HPC), an entity within the Massachusetts legislative system that already receives relatively sufficient funds, a simple

²³⁰ Herman, Bob. “A Hospital Merger with Price Caps.” *Axios*, 30 Nov. 2018, www.axios.com/2018/11/30/beth-israel-deaconess-lahey-hospital-merger-price-cap/ Accessed 3 Nov. 2025.

²³¹ “HPC Issues Final “Cost and Market Impact Review” Report for Beth Israel Lahey Health Merger | Massachusetts Health Policy Commission.” *Masshpc.gov*, 9 Aug. 2023, masshpc.gov/news/press-release/hpc-issues-final-cost-and-market-impact-review-report-beth-israel-lahey-health. Accessed 3 Nov. 2025.

²³² “Massachusetts Reviews Long-Term Effects of Major Hospital Merger – Healthcare Value Hub.” *Healthcarevaluehub.org*, 4 June 2025, healthcarevaluehub.org/state-news-roundup/massachusetts/2025/massachusetts-reviews-long-term-effects-of-major-hospital-merger/. Accessed 3 Nov. 2025.

²³³ Johnson, Gene. “After Washington State Lawsuit, Providence Health System Erases or Refunds \$158M in Medical Bills.” *AP News*, 1 Feb. 2024, apnews.com/article/ferguson-providence-medical-debt-swedish-hospital-168405fcf8525b2516a0ff571beca705.

change like this one proposes could be funded through a reallocation of funds already existent within the HPC system. For example, money could be moved out of the Health Care Repayment Reform Fund into this policy, as both policies have similar goals of increasing transparency in medical spending.²³⁴

Restriction of Major Health Mergers	Annual Cost
20 new specialists (professionals that will have strong economic and legal analysis skills) * 120K–200K	≈ \$3 million
Legislative costs (formal rulemaking, regulatory drafting, etc.)	≈ \$1 million

Enact a novel legislation in Massachusetts mirroring North Carolina’s Comprehensive Medical Debt Relief and Reform Incentive Program, entailing automatic enrollment of patients in financial assistance programs

Massachusetts legislators should enact standalone legislation aimed at alleviating patient financial burdens via the means of implementing collection limits and credit-reporting bans, mirroring North Carolina’s Comprehensive Medical Debt Relief and Reinforcement Program which serves to require hospitals to automatically screen and enroll eligible patients in charity-care programs based on income thresholds without applications. North Carolina’s program incentivizes nonprofit hospitals through state-sponsored and organized financial mechanisms, but Massachusetts has previously faced its own affordability pressures. According to the Center for Health Information and Analysis (CHIA), more than 1 in 8 Massachusetts residents carries family medical debt, even though there remains near-universal insurance coverage. CHIA also finds that low- and moderate-income households and Black residents are

²³⁴ “General Law - Part I, Title II, Chapter 6D, Section 7.” *Malegislature.gov*, 2025, malegislature.gov/Laws/GeneralLaws/PartI/TitleII/Chapter6D/Section7.

much more likely to maintain a medical debt exceeding \$2,000. Based on these documented access challenges that prevent many eligible residents from receiving hospital financial assistance, if there were an automatic-enrollment model, it would expand relief for vulnerable groups, even patients with a greater medical need, like chronic conditions, who face high out-of-pocket costs. The CHIA's findings demonstrate that if there were a streamlined financial assistance enrollment, it would be one of the most promising ways to reduce the affordability gaps that cause medical-debt hardship across the Commonwealth.

Case Study:

North Carolina's Comprehensive Medical Debt Relief and Reform Incentive Program was incredibly successful in 2025: it has already erased \$6.5 billion in medical debt for more than 2.5 million residents. Within participating nonprofit hospitals, the state's policy framework required identical financial-assistance protections for patients earning up to 300% of the federal poverty level. This supplanted the inconsistent charity-care rules that had previously varied widely depending on institution. Under the federally approved model, all 99 acute-care hospitals agreed to forgive qualifying medical debt and adopt safeguards designed to prevent new debt from accumulating. This change helped the program exceed its original projection of \$4 billion in anticipated relief by more than 60%. These reforms were supported by improved Medicaid funding that resulted in incentivizing hospitals to standardize eligibility determinations, and also work towards better compliance with charity-care obligations regarding tax-exempt status, and also to ensure that low and middle-income patients received consistent protections regardless of where they sought care, whether residing in a rural or urban community.

Cost-Benefit Analysis

The CHIA found that more than 12% (equating to over 850,000) of Massachusetts' 7.1 million residents are facing medical debt despite near universal insurance.²³⁵ Medical debt in Massachusetts correlates with over 2 billion dollars in annual collections, thereby affecting over 2.1 million residents who deal with \$2900 deductibles atop \$24000 family premiums. Non-profit hospitals maintain around \$7.5 billion in potential uncompensated care, but despite this, they surprisingly enroll less than 15% of eligibles because of application barriers.²³⁶

In Massachusetts, to implement legislation similar to North Carolina's Comprehensive Medical Debt Relief and Reform Incentive Program, automatic enrollment in charity care is estimated to cost somewhere between \$50 - 75 million due to \$15M grants from North Carolina's pilot (plus an additional \$35-60M for 80 hospitals screening 1.5M patients), and forgiving around \$1.2B in qualifying debt for around 300,000 employees at an average of \$4000 per case.²³⁷

Limitations and Implementation

This recommendation is not only permissible but very plausible, as it would take the form of new legislation rather than interfering with any existing federal or state requirements that are already mandated for nonprofit facilities. Though one challenge is accurately estimating the administrative and technology costs of building and implementing a successful, automatic, state-wide screening and enrollment architecture that can properly interface with income, tax, Medicaid, and insurance data while complying with privacy laws. This is because costs can vary with significant range depending on the hospital size and IT maturity. On another note,

²³⁵ Shure, Natalie. 2025. "More than 12 Percent of Mass. Residents Have Medical Debt, and the Number 'Will Only Grow' amid Federal Cuts | Boston Globe | November 20, 2025 – Health Care for All." Hcfama.org. 2025. <https://hcfama.org/more-than-12-percent-of-mass-residents-have-medical-debt-and-the-number-will-only-grow-amid-federal-cuts-boston-globe-november-20-2025/>.

²³⁶ "Bill H.419." 2025. Malegislature.gov. 2025. <https://malegislature.gov/Bills/194/H419.Html>.

²³⁷ "Burdened by the Bill: Understanding Medical Debt in Massachusetts Findings from the Massachusetts Health Insurance Survey." n.d. <https://www.chiamass.gov/assets/docs/r/pubs/2025/Understanding-Medical-Debt-in-Massachusetts.pdf>.

automatic enrollment might be thought of as increasing uncompensated care and reducing net patient revenue in systems that already operate on the resilience of self-pay balance collection. While expected in most recommendations, not just regarding medical debt, standardizing workflows is difficult, in this case, in the context of hospitals. There are bound to be variations in local processes and EHR systems, and even staffing can lead to uneven implementation. So while the policies might be stronger on paper, there could still be gaps that emerge that result in the persistence of gaps in financial assistance.

Regarding implementation, the first step would be for Massachusetts legislators to introduce a bill that requires hospitals to conduct automatic eligibility screening for financial assistance, prohibits extraordinary collection actions against those patients who are compliant with the thresholds, and also empowers a state agency to issue uniform standards. Massachusetts could phase in the requirements by hospital size and start with smaller, resource-constrained facilities first. It is also a necessity to partner with local health advocacy organizations, like those that have already worked on medical debt, such as the Atrus Foundation.²³⁸ That way, a patient population already involved in helping lobby for medical debt alleviation can be maintained. The following contacts can be reached: Joint Committee on Health Care Financing (House Chair: Rep. John J Lawn Jr, Senate Chair: Sen. Cindy F Friedman) for support on primary policy home for hospital transparency and finance-assistance bills; Joint Committee on Public Health (House Chair: Rep. Marjorie Decker, Senate Chair: Julian Cyr) for co-sponsoring base focused on equity House and Senate Ways and Means (Chairs: Rep. Aaron Michlewitz, Senate Michael Rodrigues) to help with any part of the fiscal portion of the bill or staffing implications at CHIA/HPC;

²³⁸ “State Protections against Medical Debt: A Look at Policies across the U.S.” 2023. Commonwealthfund.org, September. <https://doi.org/10.26099/7m6v-ve80>.

Executive agencies: CHIA (Center for Health Information and Analysis), Health Policy Commission, Attorney General's Office in the Health Care Division (Andrea Campbell).

Non-State Partnerships and Approaches

University-community Home Rehabilitation Pilot Program

Overview

1. Deliver in-home rehabilitation and recovery services to patients who lack post-hospital rehabilitation coverage, including uninsured residents currently excluded from Medicaid eligibility categories
2. Academic institutions deploy supervised teams of medical, nursing, PT/OT, and social work students providing care in partnerships with surrounding community health centers.
3. Measure and record readmission reduction, patient recovery, satisfaction, and cost savings

Low-income, recently arrived, and undocumented immigrants are ineligible for full MassHealth (Medicaid) coverage due to current federal Medicaid eligibility restrictions. This exclusion is associated with disparities in access to post-hospital recovery, rehabilitation access, and medical debt among immigrant communities, especially those residing in historically redlined and under-resourced neighborhoods such as Chelsea, Revere, and East Boston. Without comprehensive healthcare coverage, many go without necessary rehabilitation or accumulate unpayable medical debt, contributing to poorer health outcomes and the continued cycle of poverty.

Given current federal constraints on immigrant eligibility for Medicaid and related programs, this recommendation addresses state-level and community-led solutions to improve immigrant health outcomes and stabilize hospital systems.

The first step pilot program will act as an expanded health safety net given current eligibility limitations, and will provide immediate care for vulnerable populations, reduce uncompensated care costs for hospitals, and provide culturally responsible and competent care training for future clinicians. It would also set a foundation of data for later 1115 waiver applications.

Background Research

The immigrant health paradox refers to the observation that recent immigrants often have better health outcomes than individuals born in the U.S. However, the longer immigrants stay in the country, the more their health tends to decline. This deterioration has been linked to several factors, including the stress of cultural acculturation, legal challenges related to immigration status, and limited access to quality, affordable healthcare.²³⁹

In the context of Massachusetts, immigrants have higher rates of being uninsured compared to those born in the U.S. Recent 2022 data show that nearly 9% of Massachusetts immigrants are uninsured, which corresponds to approximately 49,500 individuals. These statistics do not account for the even larger number of immigrants enrolled in Limited health coverage programs, which only provide coverage for certain emergency situations. This results in numerous underinsured individuals who lack access to comprehensive preventive or specialty care, contributing to worsening health outcomes over time.²⁴⁰

²³⁹ Elshahat, Sarah, Tina Moffat, and K. Bruce Newbold. “Understanding the Healthy Immigrant Effect in the Context of Mental Health Challenges: A Systematic Critical Review.” *Journal of Immigrant and Minority Health* 24, no. 6 (2022): 1564–79. <https://doi.org/10.1007/s10903-021-01313-5>.

²⁴⁰ Health Law Advocates: Lawyers Fighting for Healthcare Justice. “Initiatives: Immigrant Health.” <https://www.healthlawadvocates.org/>.

Without access to thorough medical care, many immigrants delay treatment until conditions worsen, leading to costly emergency care and, ultimately, significant medical debt. This cycle not only erodes their initial health advantage but also places long-term financial and health burdens on immigrant communities.

Massachusetts has implemented several downstream interventions to mitigate healthcare access barriers for immigrant populations. The Massachusetts Public Accommodation Law prohibits the discrimination of health care services to anyone based on race, color, religious creed, national origin, ancestry, sex, gender identity, sexual orientation, deafness, blindness, or any physical or mental disability. Likewise, hospitals are required by state law to provide emergency service treatment to anyone, regardless of immigration or health insurance status.²⁴¹

Additional programs include MassHealth Limited, which covers emergency medical services for low-income residents regardless of immigration status;²⁴² Health Safety Net, which covers medically necessary services at participating hospitals and community health centers;²⁴³ the Children’s Medical Security Plan, which provides preventive and primary care for uninsured children;²⁴⁴ and Mass General Brigham’s Community Care Vans, which deliver mobile, culturally responsive care directly to underserved neighborhoods.²⁴⁵ While these programs provide essential emergency and short-term services, they are primarily designed to address acute care needs rather than long-term recovery.

²⁴¹ Joy Cambell, Andrea. “THE COMMONWEALTH OF MASSACHUSETTS OFFICE OF THE ATTORNEY GENERAL.” Attorney General Guidance: Information for Massachusetts Healthcare Providers Regarding Immigration Enforcement and Access to Care and Assistance Programs. <https://www.mass.gov/doc/information-for-immigrant-patients-and-their-health-care-providers-on-immigration-enforcement/download>.

²⁴² Mass.Gov. “MaassHealth Coverage Types for Individuals and Families Including People with Disabilities.” <https://www.mass.gov/info-details/masshealth-coverage-types-for-individuals-and-families-including-people-with-disabilities-0#masshealth-limited>.

²⁴³ “Health Safety Net (HSN).” Mass.gov. <https://www.mass.gov/orgs/health-safety-net>.

²⁴⁴ Health & Social Services. “Children’s Medical Security Plan.” Mass.gov. <https://www.mass.gov/childrens-medical-security-plan>.

²⁴⁵ Mass General Brigham. “Community Care Vans.” Massgeneralbrigham.org. <https://www.massgeneralbrigham.org/en/patient-care/services-and-specialties/community-care-van>.

Case Study

A useful model for the proposed University-community home rehabilitation pilot is Florida International University's Green Family Foundation Neighborhood Health Education Learning Program (NeighborhoodHelp), a large-scale, interprofessional, household-centered initiative designed to reduce health disparities in medically underserved areas in Miami-Dade County. NeighborhoodHelp assigns medical, nursing, social work, and other professional students to vulnerable households for three years, where they provide supervised social, behavioral, and clinical services while addressing the social determinants of health through continuous home visits and agreed-upon community partnerships. Throughout its first six years, 1,740 students conducted more than 7,400 household visits and delivered over 7,200 additional services through mobile health centers, leading to increased preventive service use, reductions in emergency room dependence, and over \$520,000 in legally remediated benefits for families participating in the program. This program demonstrates that student-led, community-embedded home-based care is feasible and educationally impactful by producing graduated professionals who are culturally competent and teamwork-ready. This case also shows how university teams can safely and effectively deliver in-home care to marginalized populations while simultaneously training future clinicians, providing a strong precedent for Massachusetts' proposed home rehabilitation pilot targeting uninsured, underinsured, and undocumented immigrants.²⁴⁶

Cost-Benefit analysis

University-community home rehabilitation pilot program draws on successful models such as Mass General Brigham's Home Hospital program, which delivers hospital-level care in

²⁴⁶ Greer, Pedro J., David R. Brown, Luther G. Brewster, et al. "Socially Accountable Medical Education: An Innovative Approach at Florida International University Herbert Wertheim College of Medicine." *Academic Medicine: Journal of the Association of American Medical Colleges* 93, no. 1 (2018): 60–65. <https://doi.org/10.1097/ACM.0000000000001811>.

patients' homes, including daily clinician visits, continuous vital-sign monitoring, diagnostics, medications, and therapy access. Evidence from these programs shows improved patient outcomes, reduced readmissions, and decreased inpatient bed use.²⁴⁷

While early Home Hospital implementations incur slightly higher labor and technology costs at small scales (<30–60 patients/day) due to travel and remote monitoring setup, per-patient costs stabilize at scale (~60–70 patients/day) to within ± 5 –10% of traditional inpatient care.²⁴⁸ The pilot program will focus solely on rehabilitation services, which are less resource-intensive than acute hospital care and require fewer medical devices, meaning that per-patient costs may be even lower.

Key expenditures include:

- Clinical staffing (supervised students and licensed physicians)
- Basic rehabilitation and therapy equipment
- Specialized technology for home-based care and monitoring

National Home Hospital programs estimate 19–30% per-patient cost savings compared to inpatient care. Delivering rehabilitation at home reduces post-acute facility use, prevents readmissions, and minimizes unnecessary diagnostic testing, offering significant financial and health benefits for uninsured and undocumented immigrant populations who face barriers to hospital care.²⁴⁹

Additional benefits include:

²⁴⁷ Mass General Brigham. “Research Shows Inpatient Hospital Care at Home Is Successful.” July 11, 2024. <https://www.massgeneralbrigham.org/en/about/newsroom/articles/research-shows-inpatient-hospital-care-at-home-is-successful#:~:text=The%20waiver%20allowed%20certain%20hospitals,Dementia>.

²⁴⁸ Mass General Brigham. “Home Hospital Care Demonstrates Success in Rural Communities.” <https://www.massgeneralbrigham.org/en/about/newsroom/press-releases/home-hospital-care-successful-in-rural-communities>.

²⁴⁹ AHA Center for Health Innovation Market Scan. “Providers Betting Big on Future of Hospital at Home.” <https://www.aha.org/aha-center-health-innovation-market-scan/2024-04-09-providers-betting-big-future-hospital-home>.

- Alleviating inpatient overcrowding, allowing hospitals to respond more efficiently to influx of patients
- Faster recovery and reduction of long-term complications.
- Training in culturally competent, interdisciplinary care, producing future clinicians equipped to address health disparities.

Overall, the pilot program provides a high return on investment by lowering healthcare costs, improving patient outcomes, easing hospital burden, and generating data to inform future policy and state-level health interventions.

Limitations and Implementations

The pilot program faces lots of workforce and capacity challenges. Academic institutions may not have enough students or faculty available to provide supervised home care visits at the scale required, and the need for licensed clinicians to supervise each student visit would potentially create lots of delays. Recruiting and training a multilingual, culturally competent workforce also calls for significant time and resources, which could limit the program's initial effectiveness.

Equally important is the need to build trust within immigrant communities. Recently arrived or undocumented immigrants might be hesitant to allow students or providers into their homes due to fears related to immigration enforcement, previous institutional experiences, or data privacy. These factors could slow down enrollment and reduce participation among the most vulnerable populations. To overcome these barriers, the program will have to rely heavily on partnerships with already established and trusted community health organizations, including East Boston Neighborhood Health Center, Cambridge Health Alliance, Chelsea HealthCare Center, Dorchester and Mattapan Community Health Centers, Revere Community Health Center, Boston

Medical Center, and La Colaborativa. Leveraging these trusted institutions will be critical for outreach, enrollment, and culturally sensitive care delivery, ensuring that the pilot program is accessible, accepted, and effective for the immigrant populations it is designed to serve.

Applying for Federal Section 1115 MassHealth Equity Waiver

Once there is more flexibility to test coverage approaches under Section 1115, Massachusetts could apply for a Federal Section 1115 MassHealth Equity Waiver, allowing the state to use state-only funds to expand coverage for rehabilitation services and medical debt relief for residents excluded from federal Medicaid programs due to immigration status. Section 1115 waivers grant states flexibility to test creative approaches in Medicaid that advance the objectives of the program, such as improving access, quality, and equity of care.²⁵⁰

Through this waiver, Massachusetts would transition from short-term, state-funded, safety-net expansion measures to long-term structural reform that expands healthcare access for populations, such as vulnerable immigrant communities, currently excluded from standard coverage metrics.

Background Research

Many immigrants in Massachusetts reside in historically redlined neighborhoods due to lower housing costs. Redlining originated in the 1930s when the Home Owners' Loan Corporation created color-coded maps that labeled predominantly Black, Latino, and immigrant neighborhoods as "hazardous," restricting investment and access to mortgage financing.²⁵¹

²⁵⁰ Mass.Gov. "MassHealth Section 1115 Demonstration Waiver."

<https://www.mass.gov/info-details/masshealth-section-1115-demonstration-waiver#overview>.

²⁵¹ Acevedo-Garcia, Dolores. "Redlining and Present-Day Neighborhood Opportunity in the Boston Area."

<https://www.Diversitydatakids.Org>.

<https://www.diversitydatakids.org/research-library/data-visualization/redlining-and-present-day-neighborhood-opportunity-boston-area> .

These policies resulted in long-term disinvestment in redlined neighborhoods, including fewer hospitals and clinics, limited green space, exposure to environmental hazards, and reduced access to healthy food. In modern-day Massachusetts, neighborhoods such as Roxbury, Dorchester, East Boston, Chelsea, and Revere continue to experience higher uninsurance rates, higher MassHealth enrollment, and fewer healthcare facilities compared to historically affluent neighborhoods like Back Bay.²⁵² These structural conditions concentrate health disparities and reinforce inequities in access to care.

Although Massachusetts has made meaningful progress in expanding subsidized and low-cost coverage for uninsured and underinsured residents, particularly within immigrant communities, these efforts remain insufficient to address the root causes of health disparities and medical debt. Many state programs, such as MassHealth Limited, are designed primarily to cover emergency or urgent care. As a result, individuals are often forced to delay treatment until conditions become severe or life-threatening before they can afford to seek care. Other programs, such as the Health Safety Net, allow institutions to provide medically necessary services regardless of immigration status or ability to pay; however, coverage typically ends at hospital discharge. Post-discharge needs such as rehabilitation management are not covered, leading many patients to forgo necessary care or accumulate substantial medical debt. Additionally, Health Safety Net hospitals are limited in number and are often located in urban centers or wealthier areas, far from where many low-income immigrant populations live, creating significant geographic and transportation barriers to access.²⁵³ Beginning January 1, 2026, an estimated 34,000 lawfully present immigrants in Massachusetts are projected to lose coverage

²⁵² Acevedo-Garcia, Dolores. “Redlining and Present-Day Neighborhood Opportunity in the Boston Area.” <https://www.diversitydatakids.org/research-library/data-visualization/redlining-and-present-day-neighborhood-opportunity-boston-area>.

²⁵³ Sarah L Kimball (Director, Immigrant & Refugee Health Center), in discussion with the doctor, October 2025, Boston, MA.

under the Health Connector’s Type 1 plan, further increasing uninsurance and strain on emergency departments. These unmet needs demonstrate that downstream interventions alone are insufficient and underscore the need for upstream, structural reform.

Case study

California offers a strong precedent for Massachusetts’ proposed Section 1115 MassHealth Equity Waiver through its Medi-Cal Whole Person Care (WPC) pilot and, later on, CalAIM. Launched in 2016, WPC used a Section 1115 waiver to establish post-hospital, behavioral, and community-based services for high-need Medicaid beneficiaries and uninsured populations, including undocumented immigrants. Federal Medicaid funds could not directly cover undocumented individuals, so counties instead used state and local funds under the waiver to deliver care coordination, post-discharge support, and rehabilitation-adjacent services without risking federal matching funds.

The final evaluations of WPC consistently demonstrated reduced emergency department utilization, lower hospital readmissions, and decreased uncompensated care costs for safety-net hospitals. Los Angeles County’s program, which served tens of thousands of high-risk patients, reported substantial cost avoidance by shifting care away from inpatient and emergency settings toward home-based and coordinated recovery services.²⁵⁴

The success of WPC allowed for the transition to CalAIM in 2022, which permanently embedded Enhanced Care Management and Community Supports into Medi-Cal and created broader coverage expansions for undocumented residents through the Health4All initiative. California’s experience demonstrates how Section 1115 waivers can pilot equity-focused

²⁵⁴ UCLA Center for Health Policy Research Health Economics and Evaluation Research Program. “Final Evaluation of California’s Whole Person Care (WPC) Program.” <https://www.dhcs.ca.gov/CalAIM/Documents/Final-Evaluation-of-California-Whole-Person-Care-WPC-Program-05042023.pdf>.

interventions, create cost-saving evidence, and transition short-term safety-net expansions into sustainable coverage for the most vulnerable.²⁵⁵

Cost-Benefit analysis

WPC pilot provides lots of foundational data for a Massachusetts Section 1115 MassHealth Equity Waiver focused on post-hospital rehabilitation and care coordination for immigrant populations excluded from federal Medicaid. The program operated with a \$3 billion total budget, evenly split between \$1.5 billion in state and local funds and \$1.5 billion in federal Medicaid matching funds, and used flexible payment models such as per-member-per-month bundled payments, fee-for-service reimbursements, and pay-for-outcomes incentives. These mechanisms allowed pilots to fund administrative infrastructure, home- and community-based service delivery, care coordination teams, and data systems while tying reimbursement to measurable reductions in emergency department use and hospital readmissions. Importantly, this structure allowed for services to be delivered efficiently across multiple providers without increasing overall Medicaid spending.

Overall, WPC found that enrollees experienced significantly lower acute care utilization, including 45 fewer hospitalizations and 130 fewer emergency department visits, resulting in an average annual savings of \$383 per enrollee in Medi-Cal payments. These savings were driven largely by improved care transitions, coordinated post-discharge services, and reductions in preventable emergency care, precisely the gaps faced by uninsured and undocumented immigrants in Massachusetts.²⁵⁶

²⁵⁵ Public Health Law Center at Mitchell Hamline School of Law. “EXPANDING ACCESS TO HEALTH CARE FOR ALL: CALIFORNIA SEIZES THE DAY TO SUPPORT HEALTH FOR ALL.”

<https://www.publichealthlawcenter.org/sites/default/files/resources/HCEA-California-Case-Study.pdf>.

²⁵⁶ UCLA Center for Health Policy Research Health Economics and Evaluation Research Program. “Final Evaluation of California’s Whole Person Care (WPC) Program.”

<https://www.dhcs.ca.gov/CalAIM/Documents/Final-Evaluation-of-California-Whole-Person-Care-WPC-Program-05042023.pdf>.

Limitations and Implementations

Federal approval of a Section 1115 waiver is highly dependent on the political and administrative climate, and changes in federal leadership could delay approval, limit allowable services for immigrant populations, or impose stricter budget requirements. Massachusetts would therefore need to clearly demonstrate alignment with Medicaid goals and provide evidence of cost savings and improved health outcomes, data that could be supported by findings from the proposed in-home rehabilitation pilot.

Financing presents an additional challenge, as undocumented immigrants cannot be directly covered using federal Medicaid matching funds. As a result, Massachusetts would need to rely on state-only funding sources during the initial phases of implementation, which may be difficult to get and sustain amid competing fiscal priorities.

Finally, expanding post-hospital rehabilitation and care coordination would require significant administrative capacity, new provider reimbursement models, and improved data-sharing systems across hospitals, community health centers, and rehabilitation providers. Ongoing workforce shortages, especially among rehabilitation specialists and multilingual providers, could further hinder the expansion of care, especially in historically under-resourced communities.

Massachusetts Executive Department Actions

Massachusetts Executive Agencies should set performance standards for staff in state-run medical debt programs (e.g., CICRF) that receive and evaluate applications from eligible individuals and families.

One policy recommendation to remedy the structural limitations of the workforce model of the Massachusetts Catastrophic Illness in Children Relief Fund (CICRF) is creating more operational separation of the program from the Massachusetts Department of Public Health (MADPH). By modifying the staffing model to oversee the committee on a more frequent basis under performance standards that staff have to meet, the Massachusetts CICRF can facilitate the management of resources and expedite application review time and communication processes with eligible families applying for medical reimbursements.

In terms of legislative processes, this policy recommendation can be achieved by amending Section 2a of Chapter 111K, Title XVI of Massachusetts General Law: from “established a catastrophic illness in children relief fund commission within the department of public health”²⁵⁷ to a header along the lines of “independent of the departments' supervision or control” like the description of New Jersey’s CICRF.²⁵⁸

This policy recommendation relies on the successful case of New Jersey to bridge Massachusetts in overall enhancing the access to medical relief for low-income families with children having special health needs. New Jersey’s program has not been identified with any reported issues in application processing or medical reimbursement allocation and provides double the amount of medical relief to families compared to Massachusetts.²⁵⁹ Its website

²⁵⁷ “General Law - Part I, Title XVI, Chapter 111K, Section 2.” 2025. Malegislature.gov. 2025. <https://malegislature.gov/Laws/GeneralLaws/PartI/TitleXVI/Chapter111K/Section2>.

²⁵⁸ “Catastrophic Illness in Children Relief Fund.” 2025. Catastrophic Illness in Children Relief Fund. 2025. <https://www.nj.gov/humanservices/cicrf/about/commission/>.

²⁵⁹ *New Jersey Catastrophic Illness in Children Relief Fund State Annual Report Fiscal Year 2022*. 2022.

explicitly states that the “State Office, led by the Executive Director, administers the Fund on a day-to-day basis on behalf of the Commission.” Although the structure of the commissions is nearly identical between the two, with seven public members and five state commissioners,²⁶⁰ Massachusetts CICRF’s regular staff members are all from the Department of Public Health, while New Jersey’s State Office staff are not associated with its Department of Human Services.

Furthermore, New Jersey’s State Office staff’s names and respective individual responsibilities are publicly detailed — such as fiscal manager, provider relations specialist, and program specialist — which may likely motivate their accountability and transparency. Establishing a strict performance standard can restore these qualities in the unclear performance of Massachusetts CICRF staff, to which audits in 2022 deliberately mentioned the need for “developing formal performance standards for employees, including attainable, time-based goals for processing and reviewing applications.”²⁶¹

Cost-Benefit Analysis

One specific avenue to achieve the aforementioned policy recommendation is establishing a minimum number of work hours for staff to allocate to application processing and reviews as a metric of their performance standard. This avenue of policy recommendation will help solidify accountability and incentive for staff in their efforts to process applications and hasten communications with families in a more timely manner. There is no publicly available information on the salary of Massachusetts CICRF staff, so the following cost-benefit analysis will use New Jersey CICRF staff’s salary information as a reference. This analysis is based on how much administrative costs will be expended

²⁶⁰ “Who Oversees CICRF?” 2025. Mass.gov. 2025. <https://www.mass.gov/info-details/who-oversees-cicrf>.

²⁶¹ “CICRF Did Not Process Applications in a Timely Manner.” 2018. Mass.gov. 2018. <https://www.mass.gov/info-details/cicrf-did-not-process-applications-in-a-timely-manner>.

to realize a certain amount of the objective, which is the amount of medical reimbursements or funding to applicant families in this health policy.

New Jersey Catastrophic Illness in Children Relief Fund - Fiscal Year 2023

1. Cost: Employee Wages

- a. Operating expenses: \$1,841,351 / 12 members of staff
- b. Annual salary per staff member or administrative analyst: \$153,445.92
- c. Assuming 52 weeks and 40 hours of work per week, one analyst's hourly wage: $\$153,445.92 / (52 \times 40) = \sim \74
- d. If one analyst spends 4 out of 8 hours of work every day reviewing applications, then the weekly wage spent for application review is: $74 \times 4 \times 5 = \sim \1480

2. Benefit: Funding Award for Families

- a. Number of applications received in 2023: 284
- b. Average number of applications 1 staff member can process in a year: $284/12 = 23.7$
- c. Average number of applications 1 staff member can process in a day, assuming 52 weeks and 40 hours of work per year: $23.7 / (52 \times 40) = 0.011$
- d. If one analyst spends 4 out of 8 hours of work every day reviewing applications, then the weekly average number of applications for application review is: $0.011 \times 4 \times 5 = 0.22$
- e. Average award per application: \$27,304
- f. Average award 1 staff member can process in a week: \$6006.88

Based on this analysis with New Jersey’s data, we can conclude that if the Massachusetts CICRF establishes four minimum daily work hours for an individual staff member to process and review funding applications, then the benefit-cost ratio will be around 4—meaning that applicant families can receive around an average of 4 times as much as what is spent from the CICRF to pay their staff for that time.

Massachusetts Executive Agencies Should Facilitate Automatic Notification Systems for Eligible Families Applying for State-Based Medical Debt Relief Programs and Partner with National Non-Profit Organizations.

Massachusetts executive agencies should partner with large-scale national medical debt nonprofits, such as Undue Medical Debt, to increase the scope of financial relief and communication barriers for families. The Massachusetts CICRF allocates a relatively smaller amount of medical reimbursements to eligible families compared to the CICRF in New Jersey, which collects \$0.50 more for employee medical assistance payroll tax than Massachusetts.²⁶² To increase the financial scope of the Massachusetts CICRF, a simple solution could be raising its employee medical assistance contribution rate, but the problem could be better addressed with a more sophisticated solution of partnering with nonprofit organizations to secure greater sources of funding and streamline the ease of families receiving medical reimbursements.

This policy recommendation relies on the case of Delaware in 2025, which recently partnered with a national medical debt non-profit called Undue Medical Debt to “leverage “\$500,000 in state funds to purchase and eliminate up to \$50 million in medical debt for an estimated 17,000 or more Delawareans.”²⁶³ Delaware’s partnership with Undue Medical Debt

²⁶² “Catastrophic Illness in Children Relief Fund.” Catastrophic Illness in Children Relief Fund, 2025, www.nj.gov/humanservices/cicrf/about/about-cicrf.

²⁶³ “Governor Meyer Announces Partnership for \$50 Million in Medical Debt Elimination - State of Delaware News.” 2025. State of Delaware News. July 29, 2025.

removed the application procedure for general medical relief to eligible families. Although not based on individual requests, the relief system makes it possible for qualifying residents in Delaware and partner states to automatically receive relief through the organization's purchases of debt portfolios. Eligible residents are simply notified with a letter informing them that their medical debt has been relieved without any attached taxes, penalties, or strings. This relief process happens every other week, and donations are processed in 2-3 weeks — much faster than the 300 days of current application review delay in the Massachusetts CICRF.²⁶⁴ This was made possible by how Undue Medical Debt purchases bundled medical debt portfolios in large scale from more than 300 partnered donors, such as hospitals and commercial debt buyers. Consequently, the organization has relieved \$23 billion in debt for over 14.72 million people in the US so far.²⁶⁵

As such, if the Massachusetts CICRF partners with large-scale national nonprofits that are interested in pediatric healthcare, bifold advantages of more expansive, independent staffing and greater financial portfolios can strengthen accessibility of medical relief and accelerate medical relief for families. Nonprofits can help Massachusetts leverage state funds to tackle larger medical debts through partner providers, and this expansion in medical relief will be made particularly effective when coupled with either a simplified funding application process or the absence of one, like Delaware. Hence, by securing more avenues of efficient administration and funding, the Massachusetts CICRF can address its continuing bifold issue, lacking independent, accountable staffing and having prolonged delays in communicating medical relief to families who have applied to the program. Taking these policy recommendations into consideration will

<https://news.delaware.gov/2025/07/29/governor-meyer-announce-partnership-for-50-million-in-medical-debt-elimination/>.

²⁶⁴ “CICRF Did Not Process Applications in a Timely Manner.” 2018. Mass.gov. 2018.

<https://www.mass.gov/info-details/cicrf-did-not-process-applications-in-a-timely-manner>.

²⁶⁵ “Undue Medical Debt — Donor-Powered Medical Debt Relief, Liberating Everyday People.” Undue Medical Debt. <https://unduemedicaldebt.org/>.

subsequently create meaningful progress on supporting the financial burden of children with special health needs and unlock their greater access to essential medical services.

Cost-Benefit Analysis

One way to measure the effectiveness of this second policy recommendation is determining how much Massachusetts state funds to leverage for medical debt in partnership with Undue Medical Debt or other non-profits. Delaware has leveraged costs of \$500,000 to realize the objective/benefit of medical debt as a health policy: eliminating up to \$50 million for around 17,000 Delawareans. Below is an analysis of the costs and benefits of the case.

Delaware's state funds leveraged for medical debt relief

1. Cost: \$500,000 in state funds
 - a. Delaware population: ~ 1,000,000
 - b. If we collect \$0.50 tax per 1 Delaware resident
2. Benefit: \$50,000,000 (50 million) medical debt eliminated due to partnership with Undue Medical Debt
 - a. Delaware population on medical debt (17.4%): ~174,000
 - b. Delaware population on medical debt supported by this initiative: ~17,000
 - c. Ratio of Delaware population on medical debt supported by this initiative: 0.098
 - d. ~\$2940 medical debt eliminated for 1 Delaware resident on average due to partnership with Undue Medical Debt
3. Benefit-cost ratio: For a \$0.50 tax collected per 1 Delaware resident, 5880 times as much medical debt relief as the tax can be provided to 1 Delaware resident who benefits from the partnership with Undue Medical Debt.

If we want to target the same baseline amount of medical debt relief (\$50 million) for Massachusetts residents, below are the calculations.

Determining how much Massachusetts state funds will leverage the same medical debt relief

1. Massachusetts total population: ~7.14 million
2. Massachusetts population on medical debt: 12.8%, ~914,000
3. Massachusetts population on medical debt that can be supported by nonprofit collaboration, given the same ratio that was supported by Undue Medical Debt (0.098)
 - a. $914,000 \times 0.098 = \sim 90,000$
4. If we want the same amount of medical debt, \$50,000,000, to be eliminated, \$555 medical debt will be eliminated for 1 Massachusetts resident on average.
 - a. $50 \text{ million} / 90,000 = \sim \555
5. Using the same benefit-cost ratio of Delaware, $\$555 / (5880 \text{ ratio}) = \sim \0.10 tax collected per person
6. Total state funds needed to leverage: $0.10 \times 7.14 \text{ million (total population)} = \$714,000$

Hence, Massachusetts can leverage a total of \$714,000—potentially by collecting \$0.10 tax per person in Massachusetts—to eliminate up to \$50 million medical debt when partnered with Undue Medical Debt. As this example shows, Massachusetts can therefore use similar models for calculating the costs and benefits when partnering with non-profit organizations to determine how much state funds it needs to leverage.

Limitations & Implementation

One limitation of this policy recommendation is the potential difficulty in amending the current statute on Employer Medical Assistance Contribution for the CICRF, which requires \$1 for each employee to be deposited in the CICRF. If taking the cost-benefit analysis above into

consideration, each employee will instead contribute \$1.10 to the CICRF, which would require an amendment or a new legislative clause, given the current statutes in Massachusetts. Since the Massachusetts population is around 7 times as large as Delaware's population, creating large-scale partnerships with non-profit organizations would possibly require more discussion and sensitivity across the state. Relevant legislators that could be interested in this policy recommendation are Massachusetts State Representatives Christine P. Barber and Steven Owens, who petitioned an act to address medical debt through hospital financial assistance reform in Bill H.1350.²⁶⁶

Regarding organizations, Undue Medical Debt would be ideal to reach out to; however, one limitation would be the high administrative costs of the non-profit organization that some counties have previously found difficult to meet.²⁶⁷ Alternative organizations include Dollar For and HealthWell Foundation, which are both non-profits that assist low-income patients who suffer with out-of-pocket medical costs.

²⁶⁶ "Bill H.1350." 2025. Malegislature.gov. 2025. <https://malegislature.gov/Bills/194/H1350>.

²⁶⁷ Bryant, Adrian Paul, and Beth Musgrave. "Questions Arise over Lexington's \$1 Million Contract with Medical Debt Nonprofit." Lexington Herald Leader, November 26, 2025. <https://www.kentucky.com/news/local/counties/fayette-county/article313126252.html>.